CDASH Serious Adverse Event Supplement

Version 2.0 (Final)

Developed by the
CDISC SAE Development Team

Notes to Readers

This document is based on SDTM v1.7, SDTMIG v3.3, CDASH Model v1.1, CDASHIG v2.1.

Revision History

Date	Version	Summary of changes
2021-04-21	2.0 Final	• Align document with E2B (R3) • Addition of new SA (Serious Adverse Event) CDASH domain variables
2013-11-22	1.0 Final	

See Appendix F for Representations and Warranties, Limitations of Liability, and Disclaimers.

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1 Overview of Document

1.1 Introduction

Clinical Data Acquisition Standards Harmonization (CDASH) is the standard applied to clinical data at point of capture (see https://www.cdisc.org/standards/foundational/cdash). The CDASH model contains an Adverse Event (AE) domain intended for representing information from an adverse event case report form (CRF) in the clinical data management system (CDMS).

This CDASH Serious Adverse Event (CDASH SAE) Supplement to CDASH Model Version 1.1 expands the AE domain to include data elements for capturing serious adverse event information in an SAE form and, when indicated, allows the sponsor to generate an E2B message for electronic reporting of an individual case safety report (ICSR) to health authorities. The content of an ICSR has been specified by the International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use (ICH), in the E2B(R3) specification.[1,2].

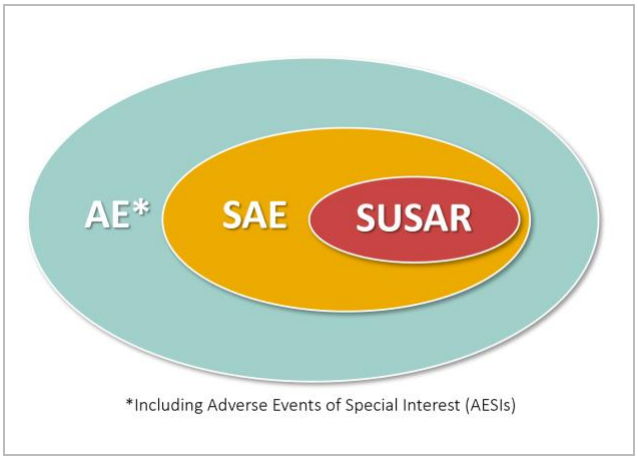
The purpose of the CDASH SAE Supplement is to facilitate collection of information that is comprehensive and relevant to understanding of the SAE and which allows for an assessment for causality. ICH E2B-compliant SAE reporting is a regulatory requirement in many countries. This supplement ensures that the safety reports sent from the site to the sponsor facilitate population of an ICH E2B-compliant SAE report without re-entry of data already collected in other areas of the data capture system. If the safety data are to be drawn from the clinical database, the sponsor must ensure that the data are collected with the appropriate level of granularity, which may be more granular than is needed for the general collection/analysis plan. The Serious Adverse Event (SAE) Tables provided in Section 2 (known as "metadata tables") illustrate the mapping of CDASH data variable names to E2B data elements. This document does not specify the exact E2B data format and should not be seen as a substitute for the guidance published by the ICH.[1]

1.2 Scope/Purpose

1.2.1 Types of Adverse Events

Clinical investigative sites typically collect data on all adverse events (AEs) during a clinical trial, but only a small subset of these will meet the criteria for a serious adverse event (SAE). Of those SAEs reported to the sponsor, a still smaller subset will meet regulatory requirements for expedited reporting to regulatory authorities. These usually are SAEs that are both unexpected and suspected of having a reasonable possibility of a causal relationship with the study drug, and are also known as suspected unexpected serious adverse reactions (SUSARs); see Figure 1.

Figure 1. Types of Adverse Events



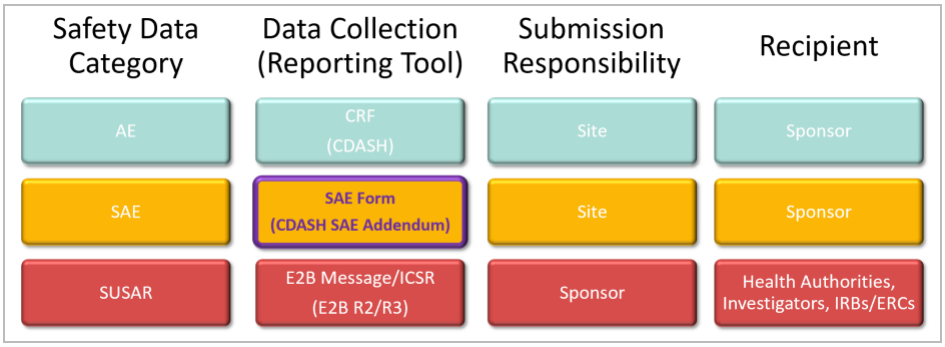
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The collection of safety information can directly impact the well-being of study subjects, either in the same study, or across studies involving the same product. Thus, certain designated safety data must be reported expeditiously by the site to the sponsor, monitoring personnel, and regulatory authorities in accordance with protocol and regulatory time frames. For any given trial, the protocol will specify the extent of safety data to be collected. Information on an adverse event will be collected and termed an AE unless it meets the regulatory and/or protocol-specified definition of an SAE. SAEs may be subject to expedited reporting to health authorities, investigators, and institutional review boards/ethics review committees (IRBs/ERCs). In general, AEs are collected on a case report form (CRF), and SAEs are collected on a specialized long form (which may be called an "SAE form"). The SAE form is used by sites to deliver required SAE data to sponsors via paper and/or the sponsor's EDC system. Note that adverse events of special interest (AESIs) are a subset of adverse events usually documented in the clinical protocol. AESIs may require additional data collection similar to that required for SAE reporting irrespective of whether they are deemed SAEs.

1.2.2 Scope of this Document

Figure 2 depicts the scope of the CDASH SAE Supplement, which is focused on the SAE form (see highlighted box under Data Collection and Reporting Tool). The SAE form is the data collection instrument for sites to record SAEs and is designed such that sponsors will receive the necessary information to report SUSARs to health authorities, investigators, and IRBs/ERCs, and to create E2B-compliant ICSRs. The existing CDASH AE domain has been expanded to include data elements to support a comprehensive SAE form. The purpose of this supplement is to ensure that all safety data (whether pertaining to AEs, SAEs, or SUSARs) employ the same data standard, and that this data standard is consistent with other CDISC data standards and with E2B specifications.

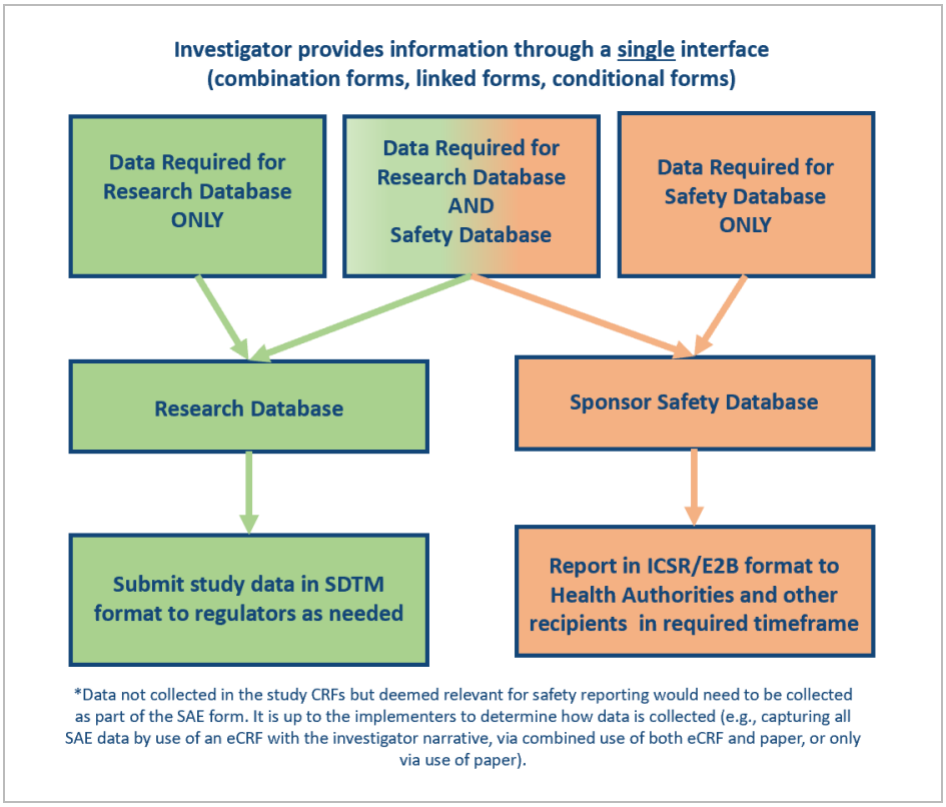
Figure 2. Levels of Event Reporting and Responsibilities



Independent of company-specific automated processes regarding data management and safety data collection, when an adverse event has occurred, the information is usually captured on an AE CRF. If an AE meets any of the SAE criteria, an SAE form is then usually completed to collect additional data on the SAE. Data that have already been collected for the study may also be needed for safety reporting and therefore is available, minimizing duplicative data entry. All of the data for SAEs are destined for a sponsor safety database, distinct from the study's research database. Examples of data already collected in existing CDASH domains (stored in the research database) and useful for typical safety case-supporting information include demographics, medical history, concomitant medications, and procedures and diagnostics. Such existing data would be available to populate the sponsor safety database (which stores SAE information), and can be used to generate safety reports as needed. See Figure 3.

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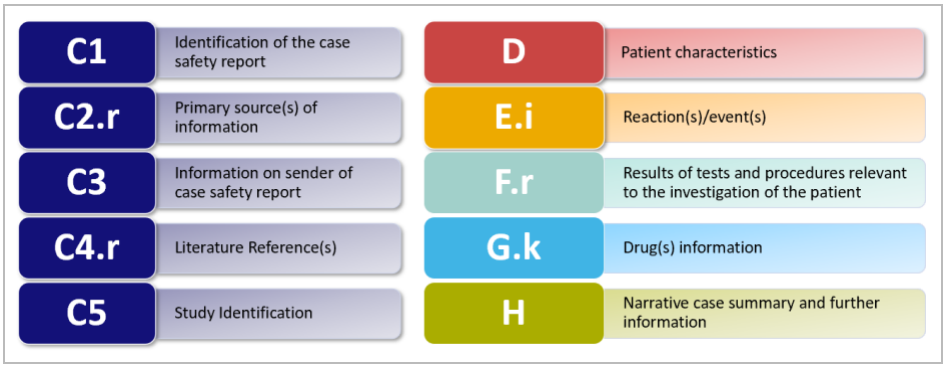
Figure 3. SAE Flow



1.2.3 Alignment with E2B

The content of the SAE form aligns with the requirements set forth in ICH E2B R3.[1] Companies can design their own SAE forms using the CDASH standard and this supplement as guidance for CDASH SAE variable names. The intent is that, at a minimum, the information provided by a site is adequate for a sponsor to submit an ICSR or E2B message to health authorities, investigators, and IRBs/ERCs. The ICSR or E2B message structure includes the subsections shown in Figure 4. This structure is used to organize the content of the SAE form (see Section 2, Serious Adverse Event (SAE) Tables). ICH E2B R3 sections are divided into sub-header sections C1-C5, and D-H, each of which has further sublevels to allow for the many specific E2B data elements. These specific E2B data elements are mapped to the CDASH variables (see Section 2). Appendix B, Data Elements from ICH E2B R2/R3, includes a high-level listing of the changes from ICH E2B R2 to R3 to assist users of this supplement in understanding the changes from R2 to R3. For a full listing of the ICH R3 E2B data elements, please refer to the ICH E2B(R3) ICSR specification.[1]

Figure 4. E2B (R3) Structure for SAE Content



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1.3 Organization of this Document

The SAE Supplement is meant to fulfill the needs of both the clinical data management and the drug safety communities. To this end, the data in Section 2, Serious Adverse Event (SAE) Tables, is organized to follow the E2B R3 guidance structured order rather than the CDASH or SDTMIG document structure.

This document contains the following sections:

- Section 1 provides an overall introduction to the scope and purpose of the CDASH E2B project.
  - Section 1.2, Scope/Purpose, provides the scope and purpose of the CDASH SAE Supplement.
  - Section 1.3, Organization of this Document, describes the organization of the CDASH SAE Supplement.
  - Section 1.4, Data Privacy Laws, provides information regarding the applicability of data privacy laws.
  - Section 1.5, CDISC Controlled Terminology, provides information regarding CDISC Controlled Terminology used in this document.
  - Section 1.6, Alignment with Other Standards, describes the relationship of the CDASH SAE Supplement to the SDTM, to CDASH Version 1.1, and to the ICH E2B specification.
  - Section 1.7, Explanation of Table Headers, describes approaches that reflect common practice implemented by a significant number of organizations and companies, defines the core designations used throughout CDASHIG v2.1 and CDASH Model 1.1, and explains the table headers used in the domain tables.
- Section 2, Serious Adverse Event Tables, contains metadata tables and recommendations for SAE data.
- Section 3, Introduction to Regulatory References, provides a list of relevant regulatory guidelines and requirements that should be consulted for instructions on the collection, organization, and submission of AE and SAE information to regulatory authorities.
- Appendices provide additional background material and describe other supplemental material relevant to the document

1.4 Data Privacy Laws

The demographic and subject identification data elements listed in this document are based on global guidelines for the collection of safety data. However, some of these data elements might be considered personal data and therefore should not be collected in cases where prohibited by local data privacy laws. Because these laws differ from country to county and are subject to change, sponsors are strongly encouraged to review the applicable data privacy laws in the country/countries in which they are planning to conduct clinical trials. Relevant fields are marked as Recommended/Conditional (R/C) to allow for adherence to local law.

1.5 CDISC Controlled Terminology

The examples in this document use CDISC Controlled Terminology where possible, but some values that seem to be controlled terminology may still be under development at the time of publication, or even especially plausible "best-guess" placeholder values. Do not rely on any source other than the CDISC value set in the National Cancer Institute Thesaurus for controlled terminology (available at http://www.cancer.gov/research/resources/terminology/cdisc).

In cases where a CDASH field has associated controlled terminology, the codelist is referenced in the Controlled Terminology column in the domain tables in this format: (codelist name}.

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1.6 Alignment with Other Standards

1.6.1 The Study Data Tabulation Model

The full set of SAE data generally is not destined for the clinical database and thus not subject to inclusion in SDTM submissions. Note also that there is an expectation that safety data are maintained in "real time" (subject to change as more information becomes available).

For information on preparing collected data for submissions, refer to the SDTM Implementation Guide (SDTMIG; available at https://www.cdisc.org/standards/foundational/sdtmig).

1.6.2 CDASHIG v2.1 and CDASH Model 1.1

A working knowledge of the CDASH standard is critical when using this document. CDASH identifies the basic data collection fields needed from a clinical, scientific, and regulatory perspective to enable more efficient and consistent data collection at investigative sites. The SDTMIG and CDASH are related: The SDTM and the SDTMIG provide a standard for the submission of data. CDASH is earlier in the data flow and defines a basic set of highly recommended and recommended or conditional data collection fields that are expected to be present on the majority of CRFs. CDASH data collection fields (or variables) facilitate mapping to SDTM domains. When data are identical between the 2 standards, the SDTMIG variable names are presented in the CDASH domain tables. In cases where the data are not identical or do not exist in the SDTMIG, CDASH has suggested new variable names, where applicable.

CDASH defines many of the basic safety data collection fields found in E2B. This CDASH SAE Supplement contains additional variables to complement existing variables in CDASH. This document intentionally does not reproduce the entire CDASH standard but rather includes existing CDASH variables that map to safety reporting data elements, where appropriate. These CDASH variables are shaded in the metadata tables. Additional data fields may need to be added to address study-specific and therapeutic-area requirements, and applicable regulatory and business practices. Because a number of CDASH variables are optional, an assessment should be made for each variable regarding inclusion on the CRF or inclusion exclusively in an E2B safety report.

1.6.3 E2B Plus

E2B Plus, or extended E2B, refers to custom XML fields (tags) that capture supplementary data beyond the ICH E2B specification.

The number and type of E2B Plus tags used varies from sponsor to sponsor and depends on many factors (e.g., study phase, therapeutic area, business need). However, there are some common E2B Plus tags such as the subject's ethnicity and the subject's hospitalization date.

E2B Plus leverages the wealth of data that is normally available during a clinical trial for a more effective safety review. For this extra data to be processed electronically by a safety system, that system itself must be able to support E2B Plus.

The CDASH SAE Supplement focuses on the ICH E2B (R3) specification and hence includes no reference to E2B Plus. **Sponsors who use the standard safety CDASHIG metadata should make sure their forms are capturing any desired extra data to be processed as E2B Plus tags.**

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1.7 Explanation of Table Headers

This section provides an overview of the table column headings used in the SAE tables in Section 2, Serious Adverse Event (SAE) Tables.

CDASH		E2B				CDASH						
1	2	3	4	5	6	7	8	9	10	11	12	13
CDASH	CDASH	E2B										
Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)

1. **CDASH Question Text:** Contains the full question text for the data collection field. Question text may be used as the label on the CRF or may be used in help text for the field.
2. **CDASH Prompt:** Contains the short prompt or label for the data collection field.
3. **E2B Variable Name:** This column provides the E2B variable names (e.g., primarysourcereaction).
4. **E2B (R2) Data Element:** This column contains the reference number of the applicable section of the ICH M2 EWG guideline for electronic transmission of ICSRs.[3]
5. **E2B (R3) Data Element:** This column contains the reference number to the applicable section of the ICH M2 EWG guideline for electronic transmission of ICSRs.
6. **E2B (R3) Data Element Name:** This column contains the E2B (R3) Data Element name from the Core Data Elements and Business Rules V1.00 Spreadsheet.
7. **CDASH Variable Name:** This column provides the SDTM, CDASH, and CDASH SAE variable names (e.g., AESPID, AEONGO, INVFNAM).
8. **DRAFT CDASH Definition:** This column defines the data to be collected for the variable. Draft Definitions have been obtained and harmonized from CDASH, SDTM, and E2B.
9. **CDISC Controlled Terminology:** In cases where a CDASH field has associated controlled terminology, the codelist is referenced in this format: (CODELIST NAME}.
10. **CDASH SAE Form Completion Instructions:** Contains information for the clinical site on how to enter collected information on the CRF.
11. **CDASH SAE Implementation Notes:** Contains further details, such as rationale and implementation instructions, for the CRF data collection fields. This information is meant to guide implementers (e.g., CRF designers, data managers, drug safety personnel).
12. **CDASH SAE Core:** Contains the CDASH Core designations for basic data collection fields. In order to facilitate classification of the different types of data collection fields, the following categories were used:
    - Highly Recommended (HR): A data collection field that should be on the CRF (e.g., a regulatory requirement).
    - Recommended/Conditional (R/C): A data collection field that should be on a CRF based on certain conditions (e.g., complete date of birth is preferred, but may not be allowed in some regions; AE time should only be captured if there is another data point with which to compare it). In the absence of this data point other data points would be less meaningful.

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- Optional (O): A data collection field that is available for use.

13. **SDTM Mapping Indicator (Y/N):** Contains a Y/N flag to indicate whether the CDASH variable has a mapping to SDTM. **Note:** Some CDASH SAE variables are only for use in transferring data from the clinical database to the safety database and therefore are not mapped to SDTM

It is assumed that sponsors will determine which data collection fields will be collected based on therapeutic area-specific data requirements, protocol, and local legal and regulatory considerations. Some data that have been previously collected may or may not be the same data as needed in the E2B message; it is up to the sponsor to confirm that these data are accurate and appropriate for use in an E2B message. For example, the country where the adverse event occurred may or may not be the same as country of the investigation site recorded in CDASH, or information already collected on CRFs may not be accurate at the time that the AE occurred (e.g., the date of last menstrual period, medical history, concomitant medications).

This document intentionally does not reproduce the entire CDASH standard but includes existing CDASH variables that map to safety reporting data elements, where appropriate. These CDASH variables are shaded in the metadata tables.

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2 Serious Adverse Event (SAE) Tables

The CDASH Adverse Event (AE) domain is intended to capture data for all adverse events, whether serious, not serious, or of special interest. The expanded SAE metadata tables combine previously collected data with additional details for a full understanding and assessment of the event and for processing in drug safety departments of sponsors.

When an event has reached the status of requiring expedited reporting to the sponsor/regulatory authority, the SAE metadata tables in this section are structured for use in conjunction with the basic data collection fields for the AE domain (as described in CDASHIG v2.1, Section 8.2.2; see https://www.cdisc.org/standards/foundational/cdash).

It is important to note that certain data in the following tables do not need to be or might not be completed by sites as indicated in the SAE form completion and implementation instructions (e.g., some data may come from other operational sources; this is dependent on how a sponsors have set up their respective data collection systems). However, these are included as a reminder of the sponsor's responsibility for information to be provided to regulatory authorities.

2.1 SAE Report Administrative and Identifier Variables (CDASH SA)

The variables included in the following table have been developed specifically for the collection of data for the identification, processing, and reporting of an SAE. This table contains fields for data collection at the site, such as information on the location of the site, study identifier, and reporter identifiers (the reporter is the primary source of the information). Note that certain data fields are included that contain information that is to be provided by the sponsor and not the investigative sites.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the site country?	Site Country	reportercountry	A.2.1.3	C.2.r.3	Reporter's Country Code	SITECO	The country for a site within a study	N/A	Record the country of the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the site's country should be auto-populated from a trial management system as part of the reporter's location information.	HR	N
What is the site postcode?	Site Postcode	reporterpostcode	A.2.1.2f	C.2.r.2.6	Reporter's Postcode	SITEPC	The postcode for a site within a study	N/A	Record the postcode of the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the site postcode should be auto-populated from a trial management system as part of the reporter's location information.	HR	N
What is the site state or province?	Site State or Province	reporterstate	A.2.1.2e	C.2.r.2.5	Reporter's State or Province	SITEST	The state or province for a site within a study	N/A	Record the state or province of the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the site's state or province should be auto-populated from a trial management system as part of the reporter's location information.	HR	N
What is the site city?	Site City	reportercity	A.2.1.2d	C.2.r.2.4	Reporter's City	SITEC	The city for a site within a study	N/A	Record the city of the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the site's city should be auto-populated from a trial management system as part of the reporter's location information.	HR	N
What is the site street address?	Site Street Address	reporterstreet	A.2.1.2c	C.2.r.2.3	Reporter's Street	SITES	The street address for a site within a study	N/A	Record the street address of the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the site's street address should be auto-populated from a trial management system as part of the reporter's location information.	HR	N

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the site telephone number?	Site Telephone Number	sendertel	A.3.1.4f	C.3.4.6	Sender's Telephone	SITETEL	The telephone number for a site within a study	N/A	Record the telephone number of the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the site's telephone number should be auto-populated from a trial management system as part of the reporter's location information.	HR	N
What is the site fax number?	Site Fax Number	senderfax	A.3.1.4i	C.3.4.7	Sender's Fax	SITEFAX	The fax number for a site within a study	N/A	Record the fax number of the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the site's fax number should be auto-populated from a trial management system as part of the reporter's location information.	R/C	N
What is the reason for SAE report nullification or amendment?	Reason for Nullification or Amendment	nullificationreason	A.1.13.1	C.1.11.2	Reason for nullification / amendment	SANULARS	The reason for serious adverse event report nullification or amendment	N/A	If case is nullified or amended, provide a reason for the change.	Only to be used if the report is nullified or amended	R/C	N
What is the date the report was submitted to the sponsor?	Sponsor Awareness Date	receivedate	A.1.6b	C.1.4	Date Report Was First Received from Source	SAAWRDAT	The date the SAE report was initially reported to the sponsor	N/A	N/A	Although this field is not typically entered on an eCRF, it should be displayed clearly on the CRF and/or the EDC system.	HR	N
What is the given name of the investigator?	Investigator Given Name	reportergivename	A.2.1.1b	C.2.r.1.2	Reporter's Given Name	INVGNAM	The first name of the primary investigator for a site	N/A	Record the first name of the principal investigator at the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, if possible, the investigator's first name should be auto-populated from a trial management system as part of the reporter's full name.	HR	N
What is the middle name of the investigator?	Investigator Middle Name	reportermiddlename	A.2.1.1c	C.2.r.1.3	Reporter's Middle Name	INVMNAM	The middle name of the primary investigator for a site	N/A	Record the middle name of the principal investigator at the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, if possible, the investigator's middle name should be auto-populated from a trial management system as part of the reporter's full name.	O	N
What is the family name of the investigator?	Investigator Family Name	reporterfamilyname	A.2.1.1d	C.2.r.1.4	Reporter's Family Name	INVFNAM	The family name of the primary investigator for a site	N/A	Record the family name of the principal investigator at the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, if possible, the investigator's family (last) name should be auto-populated from a trial management system as part of the reporter's full name.	HR	N
What is the title of the investigator?	Investigator Title	reportertitle	A.2.1.1a	C.2.r.1.1	Reporter's Title	INVTL	The title of the primary investigator for a site	N/A	Record the title of the principal investigator at the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information; as such, the investigator's title should be auto-populated from a trial management system.	HR	N
What is the email address of the investigator?	Investigator email Address	senderemailaddress	A.3.1.4l	C.3.4.8	Sender's E-mail Address	INVEMAIL	The email address of the primary investigator for a site	N/A	Record the email address of the principal investigator at the site.	The principal investigator for a site is responsible for the collection and reporting of SAE information. If E2B is generated from the EDC, the site PI's email address should be auto-populated as the sender.	O	N
What is the study identifier?	[Protocol/Study]	sponsorstudynumb	A.2.3.1	C.5.3	Sponsor Study Number	STUDYID	A unique identifier for a study	N/A	N/A	Although this field is not typically captured on a CRF, it should be displayed clearly on the CRF and/or in the EDC system.	HR	N
What is the SAE report category?	Report Category	casenullification	A.1.13	C.1.11.1	Report nullification / amendment	SAERCAT	Category of the SAE report	(SAERCAT)	Select the category of the SAE report.	This field merges the concepts previously held in NULYN so that sponsors may differentiate between Initial, amended (Follow-Up), and nullified reports.	R/C	N

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
										Depending on the reporting system, it may be derived (initial vs amendment). If a site nullifies a case, then the SANULARS variable must be populated.		
What is the subject's age at onset of the SAE?	Subject Onset Age	patientonsetage	B.1.2.2a	D.2.2a	Age at time of onset of reaction / event (number)	SAESTAGE	The age of the subject at the time of onset of the SAE, expressed in AGEU	N/A	Record the age of the subject at the time of onset of the SAE.	If possible, this field should be calculated from BRTHDAT or AGE at informed consent relative to AESTDAT. Intended for Drug Safety and not part of Demographics data for submission purposes	R/C	N
What is the subject's age unit at onset of the SAE?	Subject Onset Age Unit	patientonsetageunit	B.1.2.2b	D.2.2b	Age at time of onset of reaction / event (unit)	SAESTAGU	The unit of time that is routinely used to express the age of a person	(AGEU)	Record the appropriate age unit (e.g., YEARS, MONTHS, WEEKS, DAYS, HRS, DECADE).	Intended for Drug Safety and not part of Demographics data for submission purposes.	R/C	N
What is the subject's age group?	Subject Age Group	patientagegroup	B.1.2.3	D.2.3	Patient age group (as per reporter)	SAAGEGRP	The Age Group of the subject when more precision is not available		Record the Age Group if the person who experienced the SAE is not the subject or if the birth date of the subject was not recorded in the demographics form for the subject.	Deploy for use if subject birth dates are not collected on demographics form or if SAE data is collected in the EDC for non-subjects and date of birth is not available for those subjects. The sponsor should use and define age groups. The sponsor may use age groupings based upon the ICH submission guidelines.	O	N
What is the gestational period of the fetus at the time of observation of the SAE?	Gestational Onset Period	gestationperiod	B.1.2.2.1a	D.2.2.1a	Gestation period when reaction / event was observed in the foetus (number)	SAGESTAG	Gestational period of the fetus at time reaction/event was observed (numeric)	N/A	Enter the fetus's gestational period at the time the SAE was observed.	If the fetus is the subject of the investigation, the GESTAGE may be calculated from the value in SCORRES where SCTESTCD = EGESTAGE and need not be re-collected.	R/C	N
What is the fetal gestational period unit at the time of observation of the SAE?	Gestational Period Unit	gestationperiodunit	B.1.2.2.1b	D.2.2.1b	Gestation period when reaction / event was observed in the foetus (unit)	SAGESTGU	Unit for gestational period of the fetus at the time the reaction/event was observed	(AGEU)	Enter the fetus's gestational period unit at the time the SAE was observed.	This field is needed to complete the SAE form submitted to drug safety. This field is not intended to be part of the regulatory file submission.	R/C	N

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2.2 Patient/Subject Characteristics

In keeping with CDASH nomenclature, trial participants are referred to throughout this document as "subjects" (including patients and healthy volunteers). In E2B nomenclature, variable names using the term "patient" have already been defined by ICH and are reproduced here. Thus, in this document, "patient" and "subject" are used interchangeably to refer to clinical trial participants.

2.2.1 Subject Characteristics (CDASHIG SC)

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the subject's identifier?	Subject	patientinvestigationnumb	B.1.1.1d	D.1.1.4	Patient medical record number(s) and source(s) of the record number (investigation number)	SUBJID	A unique subject identifier within a site and study	N/A	Record the identifier for the subject.	Paper: This is typically recorded in the header of each CRF page. EDC: The subject identifiers may be provided to the site using an auto-populated list in the system. SUBJID = Subject Identifier, which must be unique within the study (often the ID of the subject as recorded on a CRF).	HR	Y
What is the estimated gestational age?	Gestational Age	N/A	N/A	N/A	N/A	EGESTAGE_SCORRES	An approximate calculation of the gestational age	N/A	N/A	If the fetus is the subject of investigation, then EGESTAGE is documented in SCORRES. If the fetus is an associated person to the subject of the investigation (parent), then EGESTAGE belong in RPORRES.	O	Y

2.2.2 Demographics (CDASHIG DM)

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the subject's date of birth?	Birth Date	patientbirthdate	B.1.2.1b	D.2.1	Date of birth	BRTHDAT	A subject's date of birth (with or without the time of birth). The complete Date of Birth is made from the temporal components of Birth Year, Birth Month, Birth Day, and Birth Time.		Record the date of birth to the level of precision known (e.g., day/month/year, year, month/year) in this format (DD-MON- YYYY).	This may be subset into the CDASH variables of BRTHDD, BRTHMO, BRTHYY, BRTHTIM as needed per specifications of the EDC system.	R/C	Y
What is the sex of the subject?	Sex	patientsex	B.1.5	D.5	Sex	SEX	Sex of the subject as determined by the investigator	(SEX)	Record the appropriate sex	Collect the subject's sex or gender, as reported by the investigator. This is a phenotypic assessment and not a genotypic assessment.	HR	Y
Which of the following 5 racial designations best describes you? (More than 1 choice is acceptable.)	Race	N/A	N/A	N/A	N/A	RACE	An arbitrary classification based on physical characteristics; a group of persons related by common descent or heredity (US Centers for Disease Control and Prevention).	(RACE)	Study participants should self-report race, with race being asked about after ethnicity.	Use RACE when the 5 designations for race used by the FDA are collected (American Indian or Alaska Native; Asian; Black or African American; Native Hawaiian or Other Pacific Islander; White). Sponsors should refer to the FDA guidance.[4]	R/C	Y
Do you consider yourself Hispanic/Latino or not Hispanic/Latino?	Ethnicity	N/A	N/A	N/A	N/A	ETHNIC	A social group characterized by a distinctive social and cultural tradition maintained from generation to generation, a common history and origin and a sense of identification with the group	(ETHNIC)	Study participants should self-report ethnicity, with ethnicity being asked about before race.	For use when values are being collected using the exact non-extensible ETHNIC codelist (C66790) values. Sponsors should refer to the FDA's guidance[4] regarding the collection of ethnicity.	R/C	Y

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2.2.3 Medical History (CDASHIG MH)

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the medical history event or concurrent condition?	Medical History Term	medicalhistoryepisode	B.1.7.2	D.7.2	Text for Relevant Medical History and Concurrent Conditions (not including reaction / event)	MHTERM	The reported or pre-specified name of the medical condition or event.	N/A	Record all relevant medical conditions or events, as defined in the protocol. Record only 1 medical condition or event per line. Ensure that the medical conditions or events listed on the Medical History page do not meet any of the exclusion criteria.	Subject's relevant medical history and concurrent conditions (e.g., disease)	HR	Y
N/A	N/A	patientepisodename	B.1.7.1a.2	D.7.1.r.1b	Medical history (disease / surgical procedure / etc.) (MedDRA code)	MHLLTCD	MedDRA Dictionary-Lowest Level Term code	N/A	N/A	Sponsors may choose to have the coding explicitly done in EDC and transfered to Safety. This field does not typically appear on the CRF. Sponsors will populate this through the coding process. This is applicable to items using MedDRA coding and must provide the MedDRA version number as well.	O	Y
What was the medical history event or concurrent condition start date?	Start Date	patientmedicalstartdate	B.1.7.1c	D.7.1.r.2	Start Date	MHSTDAT	The start date of medical history event or condition represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the start date of the medical event or condition using this format (DD-MON-YYYY).	The sponsor may choose to capture a complete date or any variation thereof (e.g., month and year).	O	Y
Is the medical history event or concurrent condition ongoing?	Ongoing	patientmedicalcontinue	B.1.7.1d	D.7.1.r.3	Continuing	MHONGO	Indication the medical condition or event is ongoing when no end date is provided	(NY)	Record the medical condition or event as ongoing ("Y") if it has not ended at the time of data collection. If the medical condition or event is ongoing, the end date should be left blank.	Completed to indicate that the condition has not resolved at the time of data collection. It is expected that every reported condition has either an End Date or the Ongoing field is populated, but not both.	O	Y
What was the medical history event or concurrent condition end date?	End Date	patientmedicalenddate	B.1.7.1f	D.7.1.r.4	End date	MHENDAT	The end date of medical history event or condition represented in an unambiguous date format (e.g., DD-MON-YYYY).	N/A	Record the end date of the medical event or condition using this format (DD-MON-YYYY).	The sponsor may choose to capture a complete date or any variation thereof (e.g., month and year, year).	O	Y

2.2.4 Relevant Procedures (CDASHIG PR)

The CDASH Procedures (PR) domain should be used to collect details describing a subject's therapeutic and diagnostic procedures conducted before, during, and/or after the study. However, the ICH E2B tags may differ if the procedure is a Medical History (D.7.1) item or if it is a procedure to diagnose the reaction/event (E2B R3 Section F.R). The following table focuses on the Medical History implementation of PR.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the procedure name?	Procedure Name	patientmedicalhistorytext	B.1.7.2	D.7.2	Text for Relevant Medical History and Concurrent Conditions (not including reaction / event)	PRTRT	The verbatim surgical, therapeutic, or diagnostic procedure's name	N/A	Record only one procedure per line.	In most cases, the verbatim procedure names or therapy will be coded to a standard dictionary (e.g., MedDRA, SNOMED) after the data have been collected on the CRF.	HR	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
N/A	N/A	patientepisodename	B.1.7.1a.2	D.7.1.r.1b	Medical history (disease / surgical procedure / etc.) (MedDRA code)	PRLLTCD	MedDRA Lowest Level Term code	N/A	N/A	This field does not typically appear on the CRF. Sponsors will populate this through the coding process.	O	Y
What was the procedure (start) date ?	(Start) Date	patientmedicalstartdate	B.1.7.1c	D.7.1.r.2	Start date	PRSTDAT	The start date of procedure represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the start date of the medical procedure using this format (DD-MON-YYYY).	The preferred method is to collect a complete start date. Partial dates (e.g., providing year only) are acceptable for procedures started a considerable amount of time prior to the start of study.	O	Y
What was the procedure (end) date?	(End) Date	patientmedicalenddate	B.1.7.1f	D.7.1.r.4	End Date	PRENDAT	The end date of procedure represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the end date of the procedure using this format (DD-MON-YYYY).	The preferred method is to collect a complete end date. Partial dates (e.g., providing year only) are acceptable for procedures started a considerable amount of time prior to the start of study.	O	Y

2.2.5 Vital Signs (CDASHIG VS)

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the result of the weight measurement?	Weight	patientweight	B.1.3	D.3	Body weight (kg)	WEIGHT_VSORRES	Subject Weight	N/A	Record the subject's weight.	Transfer to the safety database the result value closest in time to the onset of the reaction/event.	HR	Y
What was the unit of the weight measurement?	Weight Unit	patientweight	B.1.3	D.3	Body weight (kg)	WEIGHT_VSORRESU	Subject Weight Unit	(UNIT)	Record the original unit of the subject's weight	If the collected result unit is not in kilograms (kg), a conversion step will be required prior to submission to regulatory agencies.	HR	Y
What was the result of the height measurement?	Height	patientheight	B.1.4	D.4	Height (cm)	HEIGHT_VSORRES	Subject Height	N/A	Record the subject's height.	Transfer to the safety database the result value closest in time to the onset of the reaction/event.	HR	Y
What was the unit of the height measurement	Height Unit	patientheight	B.1.4	D.4	Height (cm)	HEIGHT_VSORRESU	Subject Height Unit	(UNIT)	Record the original unit of the subject's height.	If the collected result unit is not in centimeters (cm), a conversion step will be required prior to submission to regulatory agencies.	HR	Y

2.2.6 Reproductive System Findings and Parent-Child/Fetus Reporting Data (CDASHIG RP)

The RP domain is used as dictated by the study and the protocol, and the SAE form can be designed to include any additional collected data points that may provide relevant obstetric and gynecologic information migrated from the RP CRF.

This section also includes data corresponding to E2B R3 D.10 and can be used in the case of a parent-child/fetus report where the parent had no reaction/event. Refer to the E2B R3 document[1] for the relevant data elements.

Parent-child pairing information is <u>needed</u> if only the child/fetus has an adverse reaction/event (other than early spontaneous abortion/fetal demise). The characteristics concerning the parent who was the source of exposure to the drug should be provided in this section. Note that in the circumstance where both the parent and the child/fetus sustain adverse events, 2 reports are expected: 1 for the AE on the parent, and 1 for the AE on the child/fetus; these 2 separate reports should be linked.

Parent-child pairing information is <u>not needed</u> if there has been no reaction/event affecting the child/fetus (**unplanned exposure is not captured as adverse event**). Note that in the circumstance of fetal demise or early spontaneous abortion, only a parent report is applicable to detail the AE in the fetus.

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CDASHIG v2.1 discusses the principles applied to use existing CDASH metadata to collect data about people who are not subjects in the trial (e.g., the partner or child of a subject). Subject data are collected in CDASHIG variables in domains such as Demographics (DM), Medical History (MH), Adverse Events (AE), and Concomitant Medications (CM). Data specific to the child, or to a parent who is not a subject in the trial, are collected in domains such as Associated Persons (AP) Demographics (APDM), AP Medical History (APMH), and so on. The combination of subject domains and AP domains provides the appropriate variables to map to the E2B parent-infant/child report variables. In cases where the child is a study subject, data from 1 or both parents may be submitted in AP domains. In all situations linking of parent(s) to child will be described the Related Subjects (RELSUB) table as defined in the SDTM.

Reproductive System Findings

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the date of the ([subject's/partner's]) last menstrual period?	Last Menstrual Period	patientlastmenstrualdate	B.1.6.b	D.6	Last Menstrual Period Date	LMPSTDTC_RPRES	The first day of the most recent menstrual cycle.	N/A	Enter the date of first day of the ([subject's/partner's]) last menstrual period prior to the adverse event, in this format (DD-MON-YYYY).	The date of the first day of the subject's or partner's last menstrual period prior to the adverse event. Data would go to appropriate domain (RP for Reproductive Systems findings or APPR for Associated Persons Reproductive System findings). For some studies a modification of the CDASH Variable Name might be needed to differentiate PR from APPR when both are collected).	R/C	Y

Current Pregnancy

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the estimated date of conception?	Estimated Date of Conception	N/A	N/A	N/A	N/A	EDCDTC_RPRES	An approximate calculated date at which the conception event took place	N/A	Record the estimated date of conception of the current pregnancy using this format (DD-MON-YYYY).	N/A	R/C	Y
What is the estimated date of delivery?	Estimated Date of Delivery	N/A	N/A	N/A	N/A	EDLVRDTC_RPRES	An approximate calculation of the delivery date	N/A	Record the estimated date of delivery using this format (DD-MON-YYYY).	N/A	R/C	Y
What was the start date of the last menstrual period?	Last Menstrual Period Start Date	parentlastmenstrualdate	B.1.10.3b	D.10.3	Last Menstrual Period Date of Parent	LMPSTDTC_RPRES	The first day of the most recent menstrual period of the parent	N/A	Record the start date of the parent's most recent menses prior to the fetus or newborn's serious adverse event using this format (DD-MON-YYYY).	The date of the first day of the parent's last menstrual period prior to the adverse event, when the SAE is for the fetus/newborn. The "parent" may be the study subject or, if the fetus or newborn is the study subject, the parent may be an associated person.	R/C	Y
What was the date the pregnancy was confirmed?	Pregnancy Confirmation Date	N/A	N/A	N/A	N/A	PCONFIND_RPDAT	The date on which the reproductive system result or finding was collected, represented in an unambiguous date format (e.g., DD-MON-YYYY).	N/A	Record the date on which the pregnancy was confirmed, using this format (DD-MON-YYYY).	N/A	R/C	Y
Was the pregnancy confirmed?	Pregnancy Confirmed	N/A	N/A	N/A	N/A	PCONFIND_RPORRES	An indication as to whether the subject's pregnancy has been confirmed	(NY)	For the current pregnancy, indicate if the pregnancy has been confirmed.	N/A	R/C	Y

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Pregnancy History

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the total number of pregnancies (including the current, if applicable)?	Total Number of pregnancies	N/A	N/A	N/A	N/A	PREGNN_RPORRES	The number of times a woman has been pregnant, including any current pregnancy, irrespective of the pregnancy outcomes	N/A	Record the number of times the woman has been pregnant, including a current pregnancy.	N/A	R/C	Y
What is the number of ectopic pregnancies?	Number of ectopic pregnancies	N/A	N/A	N/A	N/A	ECTPREGN_RPORRES	A measurement of the total number of ectopic pregnancies experienced by a female subject	N/A	N/A	N/A	R/C	Y
What is the number of induced abortions?	Number of induced abortions	N/A	N/A	N/A	N/A	INABORTN_RPORRES	The number of surgical or medical procedure that terminates a pregnancy by removing the products of conception	N/A	Record the total number of induced abortions the woman has had.	N/A	R/C	Y
What is the number of full-term live births?	Number Full Term Live Births	N/A	N/A	N/A	N/A	BRTHFTN_RPORRES	A measurement of the total number of live birth events at which the gestational age of the neonate is 39 weeks and 0 days through 40 weeks and 6 days	N/A	N/A	N/A	R/C	Y
What is the number of late fetal deaths?	Number of Late Fetal Deaths	N/A	N/A	N/A	N/A	FETLDTHN_RPORRES	A measurement of the total number of fetal deaths (death of a fetus at 16 weeks, 0 days to 19 weeks, 6 days of gestation) experienced by a female subject	N/A	N/A	N/A	R/C	Y
What is the number of premature births?	Number of Premature Births	N/A	N/A	N/A	N/A	PREMBTHN_RPORRES	A measurement of the total number of birth events (both live and dead) at which the gestational age of the neonate is less than 37 weeks and 0 days	N/A	N/A	N/A	R/C	Y
What is the number of previous pregnancies?	Number of Previous Pregnancies	N/A	N/A	N/A	N/A	PRVPREGN_RPORRES	A measurement of the total number of pregnancy events experienced by the female subject prior to the current pregnancy	N/A	Record the number of previous pregnancies. If currently pregnant, do not include this current pregnancy in the count.	N/A	R/C	Y
What is the number of spontaneous abortions?	Number of Spontaneous Abortions	N/A	N/A	N/A	N/A	SPABORTN_RPORRES	A measurement of the total number of spontaneous abortions (in which the fetus is less than 20 weeks gestational age) experienced by a female subject	N/A	Record the number of spontaneous abortions (in which the fetus was less than 20 weeks gestational age).	N/A	R/C	Y
How many pregnancies have been carried to a viable gestational age (parity)?	Parity	N/A	N/A	N/A	N/A	PARITY_RPORRES	The number of pregnancies reaching 20 weeks and 0 days of gestation or beyond, regardless of the number of fetuses or outcomes	N/A	Record the number of pregnancies reaching 20 weeks and 0 days of gestation or beyond, regardless of the number of fetuses or outcomes	N/A	R/C	Y

Outcome of Pregnancy

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the Apgar score at 1 minute?	Apgar Score (1 Min)	N/A	N/A	N/A	N/A	APGR01_RSORRES	APGR01-Total Apgar Score	N/A	N/A	This is a standardized, scored assessment of infant development, recorded as the Total Apgar Score at 1 minute after birth. As these are scored assessments they belong in the RS domain as "Clinical Classifications"	R/C	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the Apgar score at 10 minutes?	Apgar SCore (10 Min)	N/A	N/A	N/A	N/A	APGR01_RSORRES	APGR01-Total Apgar Score	N/A	N/A	This is a standardized, scored assessment of infant development, recorded as the Total Apgar Score at 1 minute after birth. As these are scored assessments they belong in the RS domain as "Clinical Classifications"	R/C	Y
What is the Apgar score at 5 minutes?	Apgar Score (5 Min)	N/A	N/A	N/A	N/A	APGR01_RSORRES	APGR01-Total Apgar Score	N/A	N/A	This is a standardized, scored assessment of infant development, recorded as the Total Apgar Score at 1 minute after birth. As these are scored assessments they belong in the RS domain as "Clinical Classifications"	R/C	Y
What is the birth weight?	Birth Weight	N/A	N/A	N/A	N/A	BRTHWT_VSORRES	A measurement of the weight of a neonate at birth.	N/A	Record the birth weight of the newborn.	This is a vital sign of the newborn child	R/C	Y
What is the delivery date?	Delivery Date	N/A	N/A	N/A	N/A	ADLVRDTC_RPRES	The date on which the delivery procedure actually occurs.	N/A	Record the date of delivery using this format (DD-MON-YYYY).	This date is also the birth date of the child	R/C	Y
What is the pregnancy termination date?	Pregnancy Termination Date	N/A	N/A	N/A	N/A	PRGENDTC_RPRES	The date on which the pregnancy ended.	N/A	Record the date of the end of the pregnancy using this format (DD-MON-YYYY).	N/A	R/C	Y
What is the mode of delivery?	Mode of delivery	N/A	N/A	N/A	N/A	DLVRMODE_RPORRES	Description of the method by which a fetus is delivered	(MODDLV)	Record the method by which the fetus was delivered.	N/A	R/C	Y
What is the estimated gestational age?	Gestational Age	N/A	N/A	N/A	N/A	EGESTAGE_SCORRES	An approximate calculation of the gestational age	N/A	Record the calculated, estimated gestational age of the newborn.	The estimated gestational age at birth is the length of time for in utero development of the newborn, as well as the length of time of the current pregnancy	R/C	Y
What is the newborn's head circumference?	Head Circumference	N/A	N/A	N/A	N/A	FTHDCIRC_VSORRES	A circumferential measurement of the head at the widest point	N/A	Record the circumference of the head in centimeters.	Note that this result is recorded in centimeters. This value will be stored in the corresponding VSORRESU. If collecting in multiple or other units, the unit of measure should be collected separately.	R/C	Y
What was the pregnancy outcome?	Pregnancy Outcome	N/A	N/A	N/A	N/A	PRGOUT_RPORRES	Findings observed at the end of a pregnancy.	N/A	Record the narrative outcome of the pregnancy.	This is a free-text field for a narrative describing the outcome of the birth.	R/C	Y

Partner Pregnancy - Current Pregnancy

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the delivery date of the partner's pregnancy?	Partner's Delivery Date	N/A	N/A	N/A	N/A	ADLVRDTC_RPRES	(use definition from new RPTEST/CD term	N/A	Record the delivery date of the partner to the level of precision known (e.g., day/month/year, year, month/year) in this format (DD-MON-YYYY).	This is the date of delivery of the partner's pregnancy, and is also the birth date of the child.	R/C	Y
What is the number of pregnancies of the partner?	Partner Number of Pregnancies	N/A	N/A	N/A	N/A	PREGNN_RPORRES	The number of times a woman has been pregnant, including any current pregnancy, irrespective of the pregnancy outcomes	N/A	Record the number of times the woman (partner) has been pregnant, including a current pregnancy.	N/A	R/C	Y
What is the partner's number of ectopic pregnancies?	Partner Number of Ectopic Pregnancies	N/A	N/A	N/A	N/A	ECTPREGN_RPORRES	A measurement of the total number of ectopic pregnancies experienced by a female subject	N/A	N/A	N/A	R/C	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What is the partner's number of induced abortions?	Partner Number of Induced Abortions	N/A	N/A	N/A	N/A	INABORTN_RPORRES	(definition from new RPTEST/CD)	N/A	Record the total number of induced abortions the partner has had.	N/A	R/C	Y
What is the partner's number of full-term live births?	Partner Number Full Term Live Births	N/A	N/A	N/A	N/A	BRTHFTN_RPORRES	A measurement of the total number of live birth events at which the gestational age of the neonate is 39 weeks and 0 days through 40 weeks and 6 days	N/A	N/A	N/A	R/C	Y
What is the partner's number of late fetal deaths?	Partner Number Late Fetal Deaths	N/A	N/A	N/A	N/A	FETLDTHN_RPORRES	A measurement of the total number of fetal deaths (death of a fetus at 16 weeks, 0 days to 19 weeks, 6 days of gestation) experienced by a female subject	N/A	N/A	N/A	R/C	Y
What is the partner's number of premature births?	Partner Number of Premature Births	N/A	N/A	N/A	N/A	PREMBTHN_RPORRES	A measurement of the total number of birth events (both live and dead) at which the gestational age of the neonate is less than 37 weeks and 0 days	N/A	N/A	N/A	R/C	Y
What is the partner's number of previous pregnancies?	Partner Number of Previous Pregnancies	N/A	N/A	N/A	N/A	PRVPREGN_RPORRES	A measurement of the total number of pregnancy events experienced by the female subject prior to the current pregnancy	N/A	Record the number of previous pregnancies. If currently pregnant, do not include this current pregnancy in the count.	N/A	R/C	Y
What is the partner's number of spontaneous abortions?	Partner Number of Spontaneous Abortions	N/A	N/A	N/A	N/A	SPABORTN_RPORRES	A measurement of the total number of spontaneous abortions (in which the fetus is less than 20 weeks gestational age) experienced by a female subject	N/A	Record the number of spontaneous abortions (in which the fetus is less than 20 weeks gestational age).	N/A	R/C	Y
How many of the partners's previous pregnancies have reached parity?	Number of Partner pregnancies reaching parity	N/A	N/A	N/A	N/A	PARITY_RPORRES	The number of pregnancies reaching viable gestational age (including live births and stillbirths)	N/A	Record the number of pregnancies reaching 20 weeks and 0 days of gestation or beyond, regardless of the number of fetuses or outcomes.	N/A	R/C	Y

2.2.7 Past Drug History (CDASHIG CM)

This section provides a means to capture information pertaining to drugs previously taken. It is not intended for information about current drug exposure to suspect drug or concomitant medications (see Section 2.5.2, Drug Information and Other Variables). Medical judgment should be exercised regarding relevance and inclusion of this section.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the (concomitant) [medication/treatment/therapy] (name/term)?	(Concomitant) [Medication/Treatment/ Therapy]	patientdrugname	B.1.8a	D.8.r.1	Name of Drug as Reported	CMTRT	Verbatim medication name or treatments (include only treatments with data collection characteristics similar to medications)	N/A	Record only 1 [medication/treatment/therapy] per line. Provide the full trade or proprietary name of the [medication/treatment/therapy]; otherwise, the generic name may be recorded.	N/A	HR	Y
What was the individual dose of the (concomitant) [medication/treatment/therapy]?	Dose	N/A	N/A	N/A	N/A	CMDSTXT	The dose of medication/treatment taken per administration	N/A	Record the dose of (concomitant) [medication/treatment] taken per administration (e.g., 200).	N/A	O	Y
What is the unit (for the dose of concomitant [medication/treatment/therapy])?	(Dose) Unit	N/A	N/A	N/A	N/A	CMDOSU	The unit associated with the concomitant medication/treatment/therapy taken	(UNIT)	Record the dose unit of the dose of concomitant [medication/treatment/therapy] taken (e.g., mg.).	N/A	R/C	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
							(e.g., mg in "2mg three times per day")					
What was the frequency of the (concomitant) [medication/treatment/therapy]?	Frequency	N/A	N/A	N/A	N/A	CMDOSFRQ	The number of doses given/administered/taken during a specific interval	(FREQ)	Record how often the (concomitant) [medication/treatment/therapy] was taken (e.g., BID, PRN).	N/A	O	Y
What was the route of administration of the (concomitant) [medication/treatment/therapy]?	Route	N/A	N/A	N/A	N/A	CMROUTE	The route of administration of the (concomitant) [medication/treatment/therapy]	(ROUTE)	Provide the route of administration for the (concomitant) [medication/treatment/therapy].	N/A	R/C	Y
For what indication was the (concomitant) [medication/treatment/therapy] taken?	Indication	patientdrugindication	B.1.8f.2	D.8.r.6b	Indication (MedDRA code)	CMINDC	The condition, disease, symptom, or disorder that the concomitant (non-study) medication/treatment/therapy was used to address or investigate (e.g., why the medication/treatment/therapy was taken or administered).	N/A	Record the reason the medication was taken based on clinical investigator's evaluation. If taken to treat a condition, and a diagnosis was made, the indication should be the diagnosis. If taken to treat a condition, and no diagnosis was made, the indication should be the signs and symptoms. If taken as prophylaxis, report as "Prophylaxis for [description of condition]".	Note that in mapping from CMINDC (a free-text field), coding will need to be performed, as E2B (R3) uses MedDRA code values.	R/C	Y
What was the (concomitant) [medication/treatment/therapy/dose] start date?	Start Date	patientdrugstartdate	B.1.8c	D.8.r.4	Start Date	CMSTDAT	The start date when the concomitant medication/treatment/therapy was first taken, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the date the concomitant [medication/treatment] was first taken using this format (DD-MON-YYYY). If the subject has been taking the concomitant [medication/treatment] for a considerable amount of time prior to the start of the study, it is acceptable to have an incomplete date. Any concomitant [medication/treatment] taken during the study is expected to have a complete start date. Prior concomitant [medication/treatment] that is exclusionary should have both a start and end date.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD.	R/C	Y
What was the (concomitant) [medication/treatment/therapy/dose] start time?	Start Time	patientdrugstartdate	B.1.8c	D.8.r.4	Start Date	CMSTTIM	The time the concomitant medication/treatment/therapy was started, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time (as complete as possible) that the concomitant [medication/treatment] was started.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD.	R/C	Y
What was the (concomitant) [medication/treatment/therapy/dose] end date?	End Date	patientdrugenddate	B.1.8e	D.8.r.5	End Date	CMENDAT	The date that the subject ended/stopped taking the concomitant medication/treatment/therapy, represented in an unambiguous date format (e.g., DD-MON-YYYY).	N/A	Record the date the concomitant [medication/treatment] was stopped using this format (DD-MON-YYYY). If the subject has not stopped taking the concomitant [medication/treatment], leave this field blank.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD.	R/C	Y
What was the [medication/treatment/therapy/dose] end time?	End Time	patientdrugenddate	B.1.8e	D.8.r.5	End Date	CMENTIM	The time when the subject ended/stopped taking the concomitant medication/treatment/therapy, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time (as complete as possible) that the concomitant [medication/treatment] was stopped.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD.	R/C	Y

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2.2.8 Death Details (CDASHIG DD)

This section corresponds with E2B R2 B1.9 and is completed by the site or point of data collection.

Implementers are reminded that CDISC Controlled Terminology is updated quarterly. Because this document is a static publication, it refers readers to the NCI EVS page for CDISC terminology (https://datascience.cancer.gov/resources/). For the same reason, this document cannot claim to use controlled terminology in the examples provided; users should not refer to these as the ultimate authority on what terms to use or to not use.

PRCDTH_DDORRES, SECDTH_DDORRES, AUTOPIND_DDORRES, NOTESRC_DDORRES are proposed variables not included in CDASH or the SDTMIG.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the ([subject's/associated person's]) date of death?	Death date	patientdeathdate	B.1.9.1b	D.9.1	Date of Death	DTHDAT	Date of death for any subject who died	N/A	Record the date of death.	When collecting data for SAEs on clinical CRFs, it is highly recommended that death date be included. There are 2 possible variable names in the CDASH Model/IG that could be used to collect the date on which a subject died, and from which sponsors should choose: DSSTDAT or DTHDAT. Only collect the date of death once. For example, if the date of death is collected on the Death Details form, then this field would not be collected on the AE or SAE CRF module.	R/C	Y
What was the ([subject's/associated person's]) date of death?	Death Date	patientdeathdate	B.1.9.1b	D.9.1	Date of Death	DSSTDAT	Date of death for any subject who died	N/A	Record the date of death.	When collecting data for SAEs on clinical CRFs, it is highly recommended that death date be included. There are 2 possible variable names in the CDASH Model/IG that could be used to collect the date on which a subject died, and from which sponsors should choose: DSSTDAT or DTHDAT. Only collect the date of death once. For example, if the date of death is collected on the Death Details form, then this field would not be collected on the AE or SAE CRF module.	R/C	Y
What was the time of death?	Time of Death	N/A	N/A	D.9.1	Date of Death	DSSTIM	The time of the specified protocol milestone (e.g., informed consent, randomization) or disposition event (e.g., study completion or discontinuation), represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time (as complete as possible) that the subject died.	Collection of time of death is at the discretion of the sponsor.	O	Y
What was the reported primary cause of death?	Primary cause of death	patientdeathreport	B.1.9.2b	D.9.2.r.2	Reported Cause(s) of Death (free text)	PRCDTH_DDORRES	In case of death: reported cause(s) of death	N/A	Record the reported primary cause of the subject's death.	This should be populated even when the death date is unknown. The initially reported cause of death might be different than the autopsy-determined cause. For this reason, these variables may contain the same or different values. This domain is not restricted to SAE reporting. Depending upon implementation decisions made by the sponsor, death details may be	HR	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
										reported for non-adverse events (e.g., in the case of disease progression, deaths reported outside the AE collection period).		
What was the reported secondary cause of death?	Secondary cause of death	patientdeathreport	B.1.9.2b	D.9.2.r.2	Reported Cause(s) of Death (free text)	SECDTH_DDORRES	In case of death: reported cause(s) of death	N/A	Record the reported secondary cause of the subject's death.	If more than 1 secondary cause of death is collected, the sponsor might need to repeat this field using different variable names for each repeat; however, this will be system-dependent.	O	Y
Was an autopsy performed?	Autopsy performed	patientautopsyyesno	B.1.9.3	D.9.3	Was Autopsy Done?	AUTOPIND_DDORRES	In case of death: was autopsy done?	(NY)	Indicate if an autopsy was performed on the subject. If Yes, include the appropriate details where indicated on the CRF.	Autopsy is a procedure from which there will usually be findings. Autopsy information should be handled as per recommendations in the Procedures domain. Recommended values are "Y", "N", and "U".	HR	Y
What was the source of notification?	Source of Notification	N/A	N/A	N/A	N/A	NOTESRC_DDORRES	The source of information about death details	N/A	Record the source of information about the subject's death.	The source of notification or information may apply to 1 or to multiple additional death details. The CRF should be designed to allow the record generated by this collection to be grouped with all records also associated with this source. The sponsor may choose to create sponsor-defined controlled terminology for this variable. Recommended if the CRF is used to collect other death-related data such as cause of death or an indication that an autopsy was performed.	R/C	Y

2.3 SAE Event/Reaction

This section should provide details about the adverse event or reaction, such as the AE term, start and stop dates, SAE criteria, and relationship to study treatment. The variable names are used for any AE, whether serious or not serious. If an AE does not meet SAE criteria, and there is no expedited reporting requirement to regulatory authorities, the data collected will not be used to populate E2B variables.

- The variable names will have the prefix "AE" when the data are specific to all adverse events.
- The prefix "SA" is used for variables used to collect data exclusively in support of SAE reporting (i.e., never intended for SDTM submission).

Depending on the implementer, the AE and SAE variable names may be used interchangeably (e.g. AESTDAT and SASTDAT).

The metadata table in this section is structured for use in conjunction with the basic adverse event data collection fields described in the CDASHIG v2.1. The mappings to an E2B message within this table are only relevant when the adverse event reaches the threshold defined for reporting to regulatory authorities.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
Were any adverse events experienced?	Any Adverse Events	N/A	N/A	N/A	N/A	AEYN	An indication whether any AEs were experienced during the study	(NY)	Indicate if the subject experienced any adverse events. If Yes, include the appropriate details where indicated on the CRF.	This question relates to the occurrence of an adverse event, not just serious events. The intent/purpose of collecting this field is to help with data cleaning and monitoring. It provides verification	O	N

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
									At time of awareness, event may be AE or SAE (an AE that meets SAE criteria).	that all other fields on the CRF were deliberately left blank.		
[Sponsor-defined question]	[Sponsor defined]	N/A	N/A	N/A	N/A	AESPID	A sponsor-defined identifier. In CDASH, this is typically used for preprinted or auto-generated numbers on the CRF, or any other type of identifier that does not already have a defined CDASH identifier field.	N/A	If collected on the CRF, sponsors may insert instructions to ensure each record has a unique identifier.	Because SPID is a sponsor-defined identifier, conformance to Question Text or Item Prompt is not applicable. Typically used as an identifier in a data query to communicate clearly to the site the specific record in question or to reconcile concomitant medications, procedures, and/or medical history records with AEs. If CMAENO or PRAENO is used, this is the identifier to which CMAENO or PRAENO refers. May be used to record preprinted number (e.g., line number, record number) on the CRF. This field may be populated by the sponsor's data collection system.	O	Y
What is the adverse event term?	Adverse Event	primarysourcereaction	B.2.i.0	E.i.1.1a	Reaction / Event as Reported by the Primary Source in Native Language	AETERM	The reported or prespecified name of the adverse event.	N/A	Record only 1 diagnosis, sign, or symptom per line (e.g., nausea and vomiting should not be recorded in the same entry, but as 2 separate entries). Using accepted medical terminology, enter the diagnosis (if known); otherwise enter a sign or symptom. If a diagnosis subsequently becomes available, then this diagnosis should be entered on the AE form, replacing the original entries, where appropriate. Death should not be recorded as an event but should be recorded as the outcome of the event. The condition that resulted in the death should be recorded as the event unless cause of death (event leading to death) is unknown at the time of report. The report should not be delayed if unable to identify the event, in this case the report should be submitted with the adverse event recorded as “Death” or "Unknown cause of death". Once available, the event term should be replaced with the appropriate event term. Do not use abbreviations.	Can be represented either as a free text field to capture verbatim terms reported by subjects or could be preprinted in the situation where solicited AEs of interest are captured. In most cases, the verbatim term (i.e., investigator-reported term) will be coded to a standard medical dictionary such as MedDRA or WHO ART, after the data have been collected on the CRF. The coded data will be stored in field(s) not defined by CDASH. If unable to identify the event (the cause of death or event leading to death is unknown at the time of report), the report should be submitted with “death” as AE term, to be replaced with the appropriate event term once available. If the event starts out as an AE and later meets SAE criteria, AE term may need to be updated accordingly.	HR	Y
Was the adverse event serious?	Serious	serious	B.2.i.3	E.i.3.1	Term Highlighted by the Reporter	AESER	An indication whether the adverse event is determined to be serious based on regulations/protocol definition	(NY)	Assess if an adverse event should be classified as serious based on the serious criteria defined in the regulations/protocol.	This field is related to the individual SAE type fields, which may or may not be collected on the CRF. Either AESER or all the Adverse Serious type fields must be present on the CRF. The sponsor should check if the regulatory	R/C	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
										agencies to which the data will be submitted require the collection of this data.		
Did the adverse event result in death?	Death	seriousnessdeath	A.1.5.2	E.i.3.2a	Results in Death	AESDTH	An indication the serious adverse event resulted in death	(NY)	Record whether the serious adverse event resulted in death.	If details regarding SAEs are collected in the clinical database, it is recommended that a separate Yes/No variable be defined for each SAE type. Sponsors may only collect the AESER field because the individual SAE types might be collected in a separate pharmacovigilance database and therefore not collected in the clinical database. The sponsor should check if the regulatory agencies to which the data will be submitted require the collection of this data.	R/C	Y
Was the adverse event life-threatening?	Life threatening	seriousnesslifethreatening	A.1.5.2	E.i.3.2b	Life Threatening	AESLIFE	Indicates if a serious adverse event was life-threatening	(NY)	Record whether the serious adverse event is life-threatening.	If details regarding SAEs are collected in the clinical database, it is recommended that a separate Yes/No variable be defined for each SAE type. Sponsors may only collect the AESER field because the individual SAE types might be collected in a separate pharmacovigilance database and therefore not collected in the clinical database. Additional detail for the SAE report could be collected and reported as separate sponsor-defined fields or captured in the narrative.	R/C	Y
Did the adverse event result in initial or prolonged hospitalization for the subject?	Hospitalization (initial or prolonged)	seriousnesshospitalization	A.1.5.2	E.i.3.2c	Caused / Prolonged Hospitalisation	AESHOSP	An indication the serious adverse event resulted in an initial or prolonged hospitalization	(NY)	Record whether the serious adverse event resulted in an initial or prolonged hospitalization.	If details regarding SAEs are collected in the clinical database, it is recommended that a separate Yes/No variable be defined for each SAE type. Sponsors may only collect the AESER field because the individual SAE types might be collected in a separate pharmacovigilance database and therefore not collected in the clinical database. The sponsor should check if the regulatory agencies to which the data will be submitted require the collection of this data. Additional detail for the SAE report could be collected and reported as separate sponsor-defined fields or captured in the narrative.	R/C	Y
Did the adverse event result in disability or	Disability or Permanent Damage	seriousnessdisabling	A.1.5.2	E.i.3.2d	Disabling / Incapacitating	AESDISAB	An indication the serious adverse event resulted in an initial or prolonged hospitalization	(NY)	Record whether the serious adverse event resulted in a persistent or significant disability or incapacity.	If details regarding SAEs are collected in the clinical database, it is recommended that a separate Yes/No variable be defined for each SAE type.	R/C	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
permanent damage?										Sponsors may only collect the AESER field because the individual SAE types might be collected in a separate pharmacovigilance database and therefore not collected in the clinical database. The sponsor should check if the regulatory agencies to which the data will be submitted require the collection of this data. Additional detail for the SAE report could be collected and reported as separate sponsor-defined fields or captured in the narrative.		
Was the adverse event associated with a congenital anomaly or birth defect?	Congenital Anomaly or Birth Defect	seriousnesscongenitalanomali	A.1.5.2	E.i.3.2e	Congenital Anomaly / Birth Defect	AESCONG	An indication the serious adverse event was associated with a congenital anomaly or birth defect	(NY)	Record whether the serious adverse event was associated with congenital anomaly or birth defect.	If details regarding SAEs are collected in the clinical database, it is recommended that a separate Yes/No variable be defined for each SAE type. Sponsors may only collect the AESER field because the individual SAE types might be collected in a separate pharmacovigilance database and therefore not collected in the clinical database. The sponsor should check if the regulatory agencies to which the data will be submitted require the collection of this data. Additional detail for the SAE report could be collected and reported as separate sponsor-defined fields or captured in the narrative.	R/C	Y
Did the adverse event require intervention to prevent permanent impairment or damage resulting from the use of a medical product?	Needs Intervention to Prevent Impairment	seriousnessother	A.1.5.2	E.i.3.2f	Other Medically Important Condition	AESMIE	An indication an adverse event required medical or surgical intervention to preclude permanent impairment of a body function, or prevent permanent damage to a body structure, due to the use of a medical product	(NY)	Record whether the serious adverse event required intervention to prevent permanent impairment or damage due to the use of a medical product.	If details regarding SAEs are collected in the clinical database, it is recommended that a separate Yes/No variable be defined for each SAE type. Sponsors may only collect the AESER field because the individual SAE types might be collected in a separate pharmacovigilance database and therefore not collected in the clinical database. The sponsor should check if the regulatory agencies to which the data will be submitted require the collection of this data. Additional detail for the SAE report could be collected and reported as separate sponsor-defined fields or captured in the narrative.	R/C	Y
What was the date the adverse event started?	Start Date	reactionstartdate	B.2.i.4b	E.i.4	Date of Start of Reaction / Event	AESTDAT	The start date of the adverse event, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the start date of the AE using this format (DD-MON-YYYY). This is the date that the event started, even if it was not serious on this date.	The E2B variable can be used for date and time; this is specified in E2B as the date format variable B.2.i.a (YY / YYMM / YYMMDD / YYMMDDHHMM)	HR	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the adverse event start time?	Start Time	reactionstartdate	B.2.i.4b	E.i.4	Date of Start of Reaction / Event	AESTTIM	The start time of the adverse event, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the start time (as complete as possible) of the AE. This is the time that the event started, even if it was not serious at this time.	Collecting the time an AE started is only appropriate if it can be realistically determined and if there is a scientific reason for needing to know this level of detail. An example is where the subject is under the direct care of the site at the time the event started and the study design is such that it is important to know the AE start time with respect to dosing.	R/C	Y
What was the date the adverse event/reaction became serious?	Serious adverse event/ reaction start date	N/A	N/A	N/A	N/A	SASTDAT	The date when the adverse event was determined to be serious. Can be on or after the date the adverse event occurred.	N/A	Record the date when the AE became serious. Record only if different from the AE start date.	Use the standard CDASH date format. See CDASH Model v1.1 for details. This field does not map directly to a standard SDTM variable. The start date of the SAE is the date when the adverse event/reaction became serious, whether or not it has the same value as the start date of the AE. Here, SASTDAT is a separate variable from AESTDAT. Use of this field is recommended if the clinical collection tool is the primary source for collection of the data for the safety database. This may not be needed if the sponsor collects a new AE record in the clinical database when the event becomes serious.	R/C	N
What is the start time of the serious adverse event/reaction?	Serious adverse event/reaction start time	N/A	N/A	N/A	N/A	SASTTIM	Time the adverse event/reaction became serious	N/A	Enter the time the adverse event/reaction became serious.	Use the standard CDASH time format. See CDASH Model v1.1 for details. This field does not map directly (1:1) to a standard SDTM variable. The start time of the SAE is the time when the adverse event/reaction became serious, whether or not it has the same value as the start time of the AE. Here, SASTTIM is a separate variable from AESTTIM. Use of this field is recommended if the clinical collection tool is the primary source for collection of the data for the safety database. This may not be needed if the sponsor collects a new AE record in the clinical database when the event becomes serious.	O	N
What was the adverse event end date?	End Date	reactionenddate	B.2.i.5b	E.i.5	Date of End of Reaction / Event	AEENDAT	The date when the adverse event resolved/ended, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the date that the AE resolved using this format (DD-MON- YYYY). This is the date on which the event ended, regardless of whether it was still serious on this date.	The definition of resolved is sponsor-specific. The preferred method is to collect a complete end date (if applicable). Partial dates (e.g., providing year only, month and year only) may be acceptable.	HR	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
										For AEs that meet SAE criteria, the end date of the SAE is the date when the serious adverse event/reaction reached a final outcome. Here, SAENDAT is treated as a separate variable from AEENDAT; however, depending on the data policy of the implementer, AEENDAT and SAENDAT can be identical. The E2B variable can be used for date and time; this is specified in E2B as the date format variable B.2.i.a (YY / YYMM / YYMMDD / YYMMDDHHMM).		
What was the adverse event end time?	End Time	reactionenddate	B.2.i.5b	E.i.5	Date of End of Reaction / Event	AEENTIM	The time when the adverse event ended/resolved, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time (as complete as possible) that the AE resolved. This is the time at which the event ended, regardless of whether it was still serious at this time.	Collecting the time an AE resolved is only appropriate if it can be realistically determined and if there is a scientific reason for needing to know this level of detail. An example is where the subject is under the direct care of the site at the time the event resolved and the study design is such that it is important to know the AE end time with respect to dosing.	R/C	Y
Is the adverse event ongoing (as of [the study-specific time point or period])?	Ongoing (as of [the study-specific time point or period])	reactionenddate	B.2.i.5b	E.i.5	Date of End of Reaction / Event	AEONGO	Indication that an adverse event is ongoing	(NY)	Indicate if the adverse event has not resolved at the time of data collection; leave the End Date blank.	Completed to indicate that the AE has not resolved at the time of data collection, when no end date is collected. In some cases the ongoing status may be determined from AE Outcome. The purpose of collecting this field is to help with data cleaning and monitoring; this field provides further confirmation that the end date was deliberately left blank. Often used as a tick/checkbox.	O	Y
What was the end date of the serious adverse event/reaction?	Serious adverse event/reaction end date	reactionenddate	B.2.i.5b	E.i.5	Date of End of Reaction / Event	SAENDAT	Date the serious adverse event/reaction ended	N/A	Enter the date the serious adverse event/reaction ended.	The definition of resolved may be sponsor-specific. This field does not map directly to a standard SDTM variable. The end date of the SAE is the date when the serious adverse event/reaction reached a final outcome. Here, SAENDAT is treated as a separate variable from AEENDAT; however, depending on the data policy of the implementer, AEENDAT and SAENDAT can be identical.	R/C	N
What was the end time of the serious adverse event/reaction?	Serious adverse event/reaction end date	reactionenddate	B.2.i.5b	E.i.5	Date of End of Reaction / Event	SAENTIM	Time the serious adverse event/reaction ended	N/A	Enter the time the serious adverse event/reaction ended.	The definition of resolved may be sponsor-specific. This field does not map directly to a standard SDTM variable.	O	N

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
										The end time of the SAE is the time when the serious adverse event/reaction reached a final outcome. Here, SAENTIM is treated as a separate variable from SAENTIM; however, depending on the data policy of the implementer, AEENTIM and SAENTIM can be identical.		
What was the outcome of this adverse event?	Outcome	reactionoutcome	B.2.i.8	E.i.7	Outcome of Reaction / Event at the Time of Last Observation	AEOUT	A description of the outcome of an event	(OUT)	Record the appropriate outcome of the event in relation to the subject's status.	CDISC Controlled Terminology is used to indicate the outcome of the event as it relates to the subject's status. The Outcome controlled terminology includes ICH E2B values. The use of this field is the recommended way to describe whether and how the AE resolved. Because the outcome of an AE may be "Death", if this field is NOT used, be sure to provide another form, such as Disposition, with clear instructions to record deaths there.	R/C	Y
Was this adverse event related to study treatment?	Relationship to Study Treatment	drugreactionrelatedness	B.4.k.18.4	G.k.9.i.2.r.3	Result of Assessment	AEREL	An indication the study treatment had a causal effect on the adverse event, as determined by the clinician/investigator	N/A	Indicate if the cause of the adverse event is related to the study treatment and cannot be reasonably explained by other factors (e.g., subject's clinical state, concomitant therapy, other interventions).	Sponsored-defined controlled terminology is used to indicate the relationship between the AE and the study treatment (e.g., ICH E2B examples include "Not Related", "Unlikely Related", "Possibly Related", "Related"). Another possibility is the use of "Y" and "N". CDISC Controlled Terminology may be defined in the future. It is recommended that sponsors check with the appropriate regulatory authority for population of this variable to ensure it meets expectations for submission.	HR	Y
What action was taken with study treatment?	Action Taken with Study Treatment	actiondrug	B.4.k.16	G.k.8	Action(s) Taken with Drug	AEACN	Changes made to the study treatment in response to the adverse event	(ACN)	Record changes made to the study treatment resulting from the adverse event.	CDISC Controlled Terminology should be used to indicate the action taken with the study treatment in response to the AE.	HR	Y
Which event is being assessed for relatedness with the drug?	Adverse event assessed	drugreactionasses	B.4.k.18.1b	G.k.9.i.1	Reaction(s) / Event(s) Assessed	SACSLTAE	AE to which the relatedness assessment belongs	N/A	Not completed by site.	This variable links data in the SA domain to the AE domain. It is recommended that this field be auto-populated with a value from an identifier variable in AE (e.g., AELNKID, AESPID, AEREFID).	O	N
Who made this assessment?	Causality assessment source	drugassessmentsource	B.4.k.18.2	G.k.9.i.2.r.1	Source of Assessment	SACSTSCR	Source of assessment	N/A	Not completed by site.	Source of assessment (e.g., initial reporter, investigator, regulatory agency, company)	O	N
How was this assessment done?	Causality assessment method	drugassessmentmethod	B.4.k.18.3	G.k.9.i.2.r.2	Method of Assessment	SACSTMTH	Method of assessment	N/A	Not completed by site.	Method of assessment (e.g., global introspection, algorithm, Bayesian calculation)	O	N

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
Could the event have been related to/caused by the drug or study participation?	Causality assessment	drugresult	B.4.k.18.4	G.k.9.i.2.r.3	Result of Assessment	SACSLTRES	Sponsor's assessment of the relationship between the event and participating in the trial	N/A	Not completed by site.	Indicate whether the event could have been related to the drug or treatment, intervention, or study participation (e.g., procedure).	HR	N
What was the result of dechallenge?	Dechallenge Result	N/A	N/A	G.k.9.i.4	Did Reaction Recur on Re-administration?	SADCHLLT	Outcome of withdrawing interrupting, or otherwise altering administration of study treatment	(SADCRR)	Record the result of study treatment dechallenge.	Deploy in the CRF as appropriate within the sponsor's processes for collecting serious AE data. Acceptable responses are "POSITIVE", "NEGATIVE", and "NOT APPLICABLE". Dechallenge is applicable only if Action Taken with Study Drug indicates a change in dose or drug. (i.e., AEACN response cannot be "Dose Not Changed", "Not Applicable" or "Unknown").	R/C	N
What was the result of rechallenge?	Rechallenge Result	N/A	N/A	G.k.9.i.4	Did Reaction Recur on Re-administration?	SARCHLLT	Outcome of restarting or otherwise returning administration of study treatment to previous level	(SARCRR)	Record the result of study treatment rechallenge.	Deploy in the CRF as appropriate within the sponsor's processes for collecting serious AE data. Acceptable responses are "POSITIVE", "NEGATIVE", and "NOT APPLICABLE". Rechallenge is applicable only if dechallenge occurred.	R/C	N
What are the additional details of the SAE?	Narrative	narrativeincludeclinical	B.5.1	H.1	Case Narrative Including Clinical Course, Therapeutic Measures, Outcome and Additional Relevant Information	SANARR	Additional facts related to a serious adverse event	N/A	Record any additional facts not already reported that further clarify the serious event or facts surrounding it.	Deploy in the CRF as appropriate within the sponsor's processes for collecting serious AE data.	O	N

2.4 Investigations and Results

This section should capture the tests and procedures performed to diagnose or confirm the reaction/event, including tests done to investigate (exclude) a non-drug cause (e.g., serologic tests for infectious hepatitis in suspected drug-induced hepatitis). Both positive and negative results should be reported. Tests and procedures might be performed as a result of an SAE. These tests and their corresponding results might be part of the scheduled clinical data capture, but could also be recorded on an Unscheduled Visit CRF. When these tests and their results are not captured on an Unscheduled Visit CRF and mappable to an SAE report, consideration should be made for explicitly capturing these data fields on an SAE form that collects data for safety reporting.

Structured information is always preferable but, if not possible, the information can also be transmitted as free text.

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2.4.1 Information from Existing CRFs

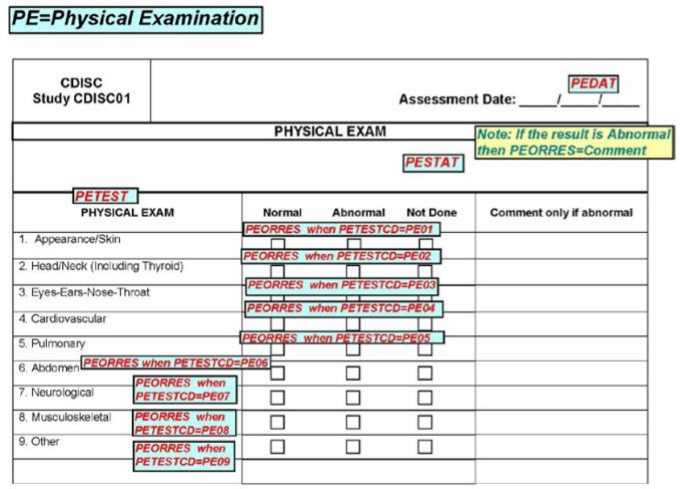
A number of results of tests and procedures fields are reported on an E2B message. The following table presents the relationship between the structured CDASH fields and the corresponding structured E2B messages for the purposes of mapping. The "--" represents the 2-character prefix of a CDASH domain and can be replaced with any Findings domain where there is relevant data, such as ECG Test Results (EG), Vital Signs (VS), Laboratory Test Results (LB), Physical Examination - option B (PE), and so on.

Results of tests and procedures relevant to the investigation of the patient:

CDASH Field[a]	E2B Data Element	E2B Title
N/A	B.3	Results of tests and procedures relevant to the investigation of the patient
N/A	B.3.1a	Structured information (repeat as necessary)
--DAT	B.3.1b	Date/Time
--TEST	B.3.1c	Test Name
--ORRES or –DESC	B.3.1d	Result
--ORRESU	B.3.1e	Unit
--ORNRHI	B.3.1.1	Normal high range
--ORNRLO	B.3.1.2	Normal low range
N/A	B.3.1.3	More information available (Y/N)
N/A	B.3.2	Free text field for results of tests and procedures relevant to the SAE of the patient that cannot be captured in the structured variables above

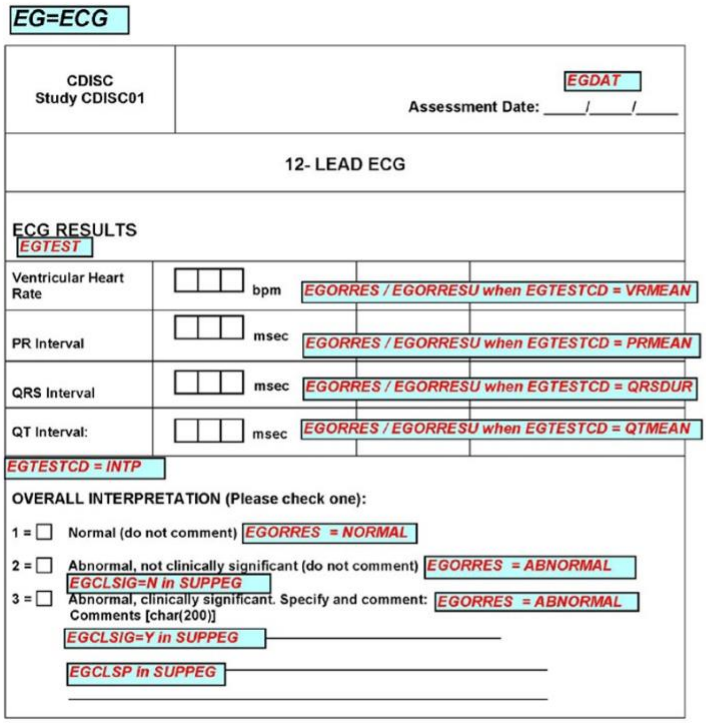
[a]N/A = not applicable; "--" represents the 2-character prefix of a CDASH domain

These mappings have been applied to the LB domain in Section 2.4.2, Laboratory Findings, as a representative approach to mapping the CDASH fields to the E2B data elements. Other findings (non-lab) could also be relevant to an event, and CRF designers should use the appropriate Findings fields from the CDASH standard for their forms. If, in response to an event, tests are performed that are not collected in standard CDASH domains, the appropriate E2B data elements should be collected on an SAE form, for purposes of safety reporting. The following are examples of non-lab findings that have existing CDASH standards (e.g., PE, EG), and the mapping of the CDASH variable names to the E2B Data Elements.



CDASH Variable Name	E2B Data Element	E2B Title
PEDAT*	B.3.1b	Date/Time
PETEST	B.3.1c	Test Name
PEORRES	B.3.1d	Result

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CDASH Variable Name	E2B Data Element	E2B Title
EGDAT	B.3.1b	Date/Time
EGTEST	B.3.1c	Test Name
EGORRES	B.3.1d	Result

2.4.2 Laboratory Findings (CDASH LB)

This section should capture the laboratory tests and procedures performed to diagnose or confirm the reaction/event, including tests done to investigate (exclude) a non-drug cause, (e.g., serologic tests for infectious hepatitis in suspected drug-induced hepatitis). Both positive and negative results should be reported (B.3.1). Although structured information is preferable, provisions have been made to transmit the information as free text in B.3.2.

Note: Although this metadata table is explicitly for the LB domain, there are other "lab-like" domains which might have relevant lab tests and results, for purposes of populating the E2B message, including Pharmacokinetics Concentrations/Parameters (PC/PP), Microbiology Specimen/Susceptibility (MB/MS), Immunogenicity Specimen Assessments (IS), and Microscopic Findings (MI). Any of the lab-like domains might have data relevant for reporting in the SAE E2B message, and should be considered for inclusion as part of the diagnostic lab data.

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the lab test name?	[Laboratory Test Name]	testname	B.3.1c	F.r.2.1	Test Name (free text)	LBTEST	Descriptive name of the lab test or examination used to obtain the measurement or finding. Any test normally performed by a clinical laboratory is considered a lab test.	(LBTEST)	Record the name of the lab measurement or finding, if not preprinted on the CRF. If collected on the CRF, the sponsor may provide additional instructions to ensure the data is entered as intended.	N/A	HR	Y
What was the result of the lab test?	(Result)	testresult	B.3.1d	F.r.3.2	Test Result (value / qualifier)	LBORRES	Result of the measurement or finding as originally received or collected	N/A	Record the laboratory test result.	N/A	HR	Y
What was the unit of the lab result?	Unit	testunit	B.3.1e	F.r.3.3	Test Result (unit)	LBORRESU	The unit of the result as originally received or collected	(UNIT)	Record or select the original unit in which these data were collected, if not preprinted on CRF.	When mapping, note that E2B R3 terminology applies the UCUM (Unified Code for Units of Measure)	R/C	Y
What was the date of the lab specimen collection	Collection Date	testdate	B.3.1b	F.r.1	Test Date	LBDAT	The date of specimen collection, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the date when the specimen collection was done, using this format (DD-MON-YYYY).	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What was the (start) time of the lab specimen collection?	[Start] Collection Time	testdate	B.3.1b	F.r.1	Test Date	LBTIM	The time of specimen collection, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time of collection (as complete as possible).	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What [is/was] the specimen (material) type?	Specimen (Material) Type	N/A	N/A	N/A	N/A	LBSPEC	The type of specimen used for a measurement	(SPECTYPE)	N/A	N/A	O	Y
What was the high limit of the reference range for this lab test?	Normal Range Upper Limit	hightestrange	B.3.1.2	F.r.5	Normal High Value	LBORNRHI	The upper end of normal range or reference range for continuous results stored in LBORRES	N/A	Record the upper limit of the reference range of the lab test.	N/A	R/C	Y
What was the lower limit of the reference range for this lab test?	Normal Range Lower Limit	lowtestrange	B.3.1.1	F.r.4	Normal Low Value	LBORNRLO	The lower end of normal range or reference range for continuous results stored in LBORRES	N/A	Record the lower limit of the reference range of the lab test.	N/A	R/C	Y

2.5 Drug Information

This section describes the collection of information about a suspect drug that might be a concomitant medication or study treatment.

2.5.1 Information from Existing Drug Information CRFs

Implementers should use the fields identified in the CDASH standard. The fields listed in this section include the CDASH field and additional fields that can be used if needed. (See Section 1.2, Scope/Purpose, for a more detailed explanation.) This section relates to drug data that might have been captured in the Exposure as Collected (EC) or the Concomitant Medication (CM) domain via existing CDASH forms. If the suspect drug is a concomitant medication, then the applicable fields from the CM domain are completed. If the suspect drug is study treatment, then the applicable fields from the EC domain are completed. Variables from both domains have been included for purposes of completeness, recognizing that the identified drugs would probably only come from 1 of these identified sources. Note that some sponsors might use the Exposure (EX) domain as the primary domain for initial exposure data collection, instead of Exposure as Collected. Please refer to CDASH and the CDASHIG (available at https://www.cdisc.org/standards/foundational/cdash) to understand the relationship between these related domains.

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2.5.2 Drug Information (CDASH EC, CDASH CM) and Other Variables Associated with Suspect Drug

This section covers both suspect drugs and concomitant medication. If the event is related to study treatment(s), the associated variables for the EC domain are populated. If the event is related to concomitant treatment(s), the associated variables in the CM domain are populated. In addition, the section can be used to identify drugs thought to have an interaction. If the event results from the interaction between study treatment(s) and concomitant therapy(ies), then the associated variables for both CM and EC domains are populated. Drugs used to treat the reaction/event should be recorded in the CM domain.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
Was [study treatment/dose] administered?; Has the subject taken [study treatment/dose]?	[Study Medication/Treatment]	N/A	N/A	N/A	N/A	ECOCCUR	An indication whether the study treatment was administered when information about the occurrence of a specific intervention is solicited	(NY)	Indicate if the subject took study treatment. If Yes, include the appropriate details where indicated.	N/A	O	Y
What was the [study treatment/investigational product] name?	[Study Treatment/Investigational Product Name]	medicinalproduct	B.4.k.2.1	G.k.2.2	Medicinal Product Name as Reported by the Primary Source	ECTRT	Name of the intervention or treatment known to the subject and/or administrator	N/A	Record the name of study treatment.	N/A	R/C	Y
What was the dose (per administration) (of [study treatment/dose])?	Dose	drugstructuredosagenumb	B.4.k.5.1	G.k.4.r.1a	Dose (number)	ECDSTXT	The dose of study medication taken (per administration)	N/A	Record the dose or amount of study treatment that was administered to/taken by the subject in the period recorded; from the start date/time to the end date/time inclusive.	N/A	R/C	Y
What were the units for the dose?	Units	drugstructuredosageunit	B.4.k.5.2	G.k.4.r.1b	Dose (unit)	ECDOSU	The unit for intended dose (per administration) for ECDOSE, ECDOSTOT, or ECDOSTXT	(UNIT)	Record the unit of dose or amount taken per period recorded (e.g., ng, mg, mg/kg).	N/A	R/C	Y
What was the frequency of [study treatment/dose] dosing?	Frequency	drugseparatedosagenumb; drugintervldosageunitnumb; drugintervaldosagedefinition	B.4.k.5.3; B.4.k.5.4; B.4.k.5.5	N/A; G.k.4.r.2; G.k.4.r.3	Number of Units in the Interval; Definition of the Time Interval Unit	ECDOSFRQ	The number of doses given/administered/taken during a specific interval	(FREQ)	Record the number of doses given/administered/taken during a specific interval.	The single CDASH variable maps to more than one E2B variable. This mapping is out of scope for data collection and must be managed operationally.	R/C	Y
What was the lot number of the [study treatment/dose] used?	Lot Number	drugbatchnumb	B.4.k.3	G.k.4.r.7	Batch / Lot Number	ECLOT	Lot Number of the ECTRT product	N/A	Record the lot number that appears on the container holding the study treatment.	N/A	R/C	Y
What was the anatomical location of the ([study treatment/dose]) administration?	Anatomical Location	N/A	N/A	N/A	N/A	ECLOC	A description of the anatomical location of administration	(LOC)	Record the body location where the study treatment was administered (e.g., SHOULDER, HIP, ARM).	N/A	O	Y
What was the ([intended/planned/actual]) ([study treatment/dose]) (start) date?	(Start) Date	drugstartdate	B.4.k.12b	G.k.4.r.4	Date and Time of Start of Drug	ECSTDAT	The start date of study treatment, intended or actual, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the start date of the study treatment administration using this format (DD-MON-YYYY).	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	HR	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the ([intended/planned/actual]) ([study treatment/dose]) (start) time?	(Start) Time	drugstartdate	B.4.k.12b	G.k.4.r.4	Date and Time of Start of Drug	ECSTTIM	The start time of study treatment, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the start time (as complete as possible) when administration of study treatment started.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What was the ([intended/planned/actual]) ([study treatment/dose]) (end) date?	(End) Date	drugenddate	B.4.k.14b	G.k.4.r.5	Date and Time of Last Administration	ECENDAT	The end date of study treatment, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the end date of the study treatment administration using this format (DD-MON-YYYY).	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What was the ([intended/planned/actual]) ([study treatment/dose]) (end) time?	(End) Time	drugenddate	B.4.k.14b	G.k.4.r.5	Date and Time of Last Administration	ECENTIM	The end time of study treatment, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time (as complete as possible) when study treatment administration stopped (e.g., for infusions this is the time when the infusion ended).	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What was the (concomitant) [medication/treatment/therapy] (name/term)?	(Concomitant) [Medication/Treatment/Therapy]	medicinalproduct	B.4.k.2.1	G.k.2.2	Medicinal Product Name as Reported by the Primary Source	CMTRT	Verbatim medication name or treatments (include only treatments with data collection characteristics similar to medications)	*N/A*	Record only 1 [medication/treatment/therapy] per line. Provide the full trade or proprietary name of the [medication/treatment/therapy]; otherwise, the generic name may be recorded.	N/A	HR	Y
What was the individual dose of the (concomitant) [medication/treatment/therapy]?	Dose	drugstructuredosagenumb	B.4.k.5.1	G.k.4.r.1a	Dose (number)	CMDSTXT	The dose of medication/treatment taken per administration	N/A	Record the dose of (concomitant) [medication/treatment] taken per administration (e.g., 200).	Implementers might choose to use CMDOS, instead of using the text field of CMDSTXT, if the collected dose will always be numeric.	O	Y
What is the unit (for the dose of concomitant [medication/treatment/therapy])?	(Dose) Unit	drugstructuredosageunit	B.4.k.5.2	G.k.4.r.1b	Dose (unit)	CMDOSU	The unit associated with the concomitant medication/treatment/therapy taken (e.g., mg in "2mg three times per day")	(UNIT)	Record the dose unit of the dose of medication taken (e.g., mg.).	N/A	R/C	Y
What was the frequency of the (concomitant) [medication/treatment/therapy]?	Frequency	drugseparatedosagenumb; drugintervldosageunitnumb; drugintervaldosagedefinition	B.4.k.5.3; B.4.k.5.4; B.4.k.5.5	N/A; G.k.4.r.2; G.k.4.r.3	Number of Units in the Interval; Definition of the Time Interval Unit	CMDOSFRQ	The number of doses given/administered/taken during a specific interval	(FREQ)	Record how often the (concomitant) [medication/treatment/therapy] was taken (e.g., BID, PRN).	N/A	O	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the route of administration of the (concomitant) [medication/treatment/therapy]?	Route	drugadministrationroute	B.4.k.8	G.k.4.r.10.1	Route of Administration (free text)	CMROUTE	The route of administration of the (concomitant) [medication/treatment/therapy]	(ROUTE)	Provide the route of administration for the (concomitant) [medication/treatment/therapy].	N/A	R/C	Y
For what indication, was the (concomitant) [medication/treatment/therapy] taken?	Indication	N/A	N/A	G.k.7.r.1	Indication as Reported by the Primary Source	CMINDC	The condition, disease, symptom, or disorder that the concomitant (non-study) medication/treatment/therapy was used to address or investigate (e.g., why the medication/treatment/therapy was taken or administered)	N/A	Record the reason the medication was taken based on clinical investigator's evaluation. If taken to treat a condition, and a diagnosis was made, the indication should be the diagnosis. If taken to treat a condition, and no diagnosis was made, the indication should be the signs and symptoms. If taken as prophylaxis, report as "Prophylaxis for [description of the condition(s)]".	N/A	R/C	Y
What was the (concomitant) [medication/treatment/therapy/dose] start date?	Start Date	drugstartdate	B.4.k.12b	G.k.4.r.4	Date and Time of Start of Drug	CMSTDAT	The start date when the concomitant medication/treatment/therapy was first taken, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the date the concomitant [medication/treatment] was first taken using this format (DD-MON-YYYY). If the subject has been taking the concomitant [medication/treatment] for a considerable amount of time prior to the start of the study, it is acceptable to have an incomplete date. Concomitant [medication/treatment] taken during the study are expected to have a complete start date. Prior concomitant [medication/treatment] that are exclusionary should have both a start and end date.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What was the (concomitant) [medication/treatment/therapy/dose] start time?	Start Time	drugstartdate	B.4.k.12b	G.k.4.r.4	Date and Time of Start of Drug	CMSTTIM	The time the concomitant medication/treatment/therapy was started, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time (as complete as possible) that the concomitant [medication/treatment] was started.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What was the (concomitant) [medication/treatment/therapy/dose] end date?	End Date	drugenddate	B.4.k.14b	G.k.4.r.5	Date and Time of Last Administration	CMENDAT	The date that the subject ended/stopped taking the concomitant medication/treatment/therapy, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the date the concomitant [medication/treatment] was stopped using this format (DD-MON-YYYY). If the subject has not stopped taking the concomitant [medication/treatment], leave this field blank.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y
What was the [medication/treatment/therapy/dose] end time?	End Time	drugenddate	B.4.k.14b	G.k.4.r.5	Date and Time of Last Administration	CMENTIM	The time when the subject ended/stopped taking the concomitant medication/treatment/therapy, represented in an unambiguous time format (e.g., hh:mm:ss)	N/A	Record the time (as complete as possible) that the concomitant medication/treatment was stopped.	Mapping of CDASH date and time variables needs to be done to achieve the E2B (R3) format of CCYYMMDD	R/C	Y

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
Is there any other relevant information on the drug?	Additional drug information	drugadditional	B.4.k.19	G.k.11	Additional Information on Drug (free text)	SADRGADD	Free-text field for additional information on drug	N/A	Enter any other relevant information on the drug (e.g., beyond expiration date, batch and lot tested and found to be within specifications).	This should be used to specify any additional information pertinent to the case that is not covered by preceding sections. This item can also be used to provide additional information concerning the indication for the drug.	O	N

2.6 Narrative

The objective of the narrative is to summarize all relevant clinical and related information, including subject characteristics; therapy details; medical history; clinical course of the event(s); and diagnosis, including the outcome, laboratory evidence, and any other information that supports or refutes an adverse drug reaction. The narrative should serve as a comprehensive, stand-alone "medical story." The information should be presented in a logical time sequence; ideally, this should be presented in the chronology of the subject/patient's experience, rather than the chronology in which the information was received.

Key information from supplementary records should be included in the report. Any relevant autopsy or postmortem findings should also be summarized in the narrative. Terms (e.g., AEs/SAEs, indication, medical conditions) in the narrative should be accurately reflected in appropriate data fields.

The purpose of the medical review is to ensure correct interpretation of the medical information. The review should include, but is not limited to, the following considerations:

- Is a diagnosis possible?
- Have the relevant diagnostic procedures been performed?
- Were alternative causes of the event/reaction(s) considered?
- What additional information may be needed to allow for the final conclusion regarding whether the SAE is caused by the product? Include the rationale for the final conclusion

The following table provides the high-level structure of the Narrative and Assessment section of the SAE form. See Appendix E for content elements of the narrative and a sample narrative template.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What are the details of the SAE/reaction?	SAE Narrative	narrativeincludeclinical	B.5.1	H.1	Case Narrative Including Clinical Course, Therapeutic Measures, Outcome and Additional Relevant Information	SANARR	Additional facts related to a serious adverse event	N/A	Include these sections: • Description of the SAE • Description of the work-up and evaluation therapeutic measures taken to treat the SAE	Summarize the details relevant to the case with enough information to allow for a full understanding of the case, and to perform a causality assessment.	R/C	N

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CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
									• Outcome of the SAE • Additional relevant information	The reporter (investigator) presents their findings and assessment in the SAE report to the sponsor. The sender (sponsor) has corresponding sections for their comments and assessment, captured in H.3.r.		
What is the rationale for the final assessment for causality?	Rationale for Final Investigator Causality	reportercomment	B.5.2	H.2	Reporter's Comments	SAIRATNL	Rationale for final investigator causality	N/A	Provide the conclusion of the medical review and the determination of causality. Include the rationale for the conclusion.	This should be a clear, concise statement as to whether or not the product caused the SAE, and the reason(s) why or why not.	R/C	N

2.7 Hospitalization Supplemental (CDASHIG HO)

This section provides a means to capture additional hospitalization information that may be required to be transferred to the safety database to evaluate the SAE case.

CDASH Question Text	CDASH Prompt	E2B Variable Name	E2B (R2) Data Element	E2B (R3) Data Element	E2B (R3) Data Element Name	CDASH Variable Name	DRAFT CDASH Definition	CDISC Controlled Terminology	CDASH SAE Form Completion Instructions	CDASH SAE Implementation Notes	CDASH SAE Core	SDTM Mapping Indicator (Y/N)
What was the hospital admission date?	Hospital Admission Date	N/A	N/A	N/A	N/A	HOSTDAT	The start date the healthcare encounter, represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the start date of the healthcare encounter (e.g., date of admission) using this format (DD-MON-YYYY).	The preferred method is to collect a complete start date. Partial dates (e.g., providing year only, month and year only) may be acceptable. Collecting this SAE-related data in the clinical database is the decision of the sponsor.	R/C	Y
What was the hospital discharge date?	Hospital Discharge Date	N/A	N/A	N/A	N/A	HOENDAT	The end date of the healthcare encounter, (date of discharge) represented in an unambiguous date format (e.g., DD-MON-YYYY)	N/A	Record the end date of the healthcare encounter using this format (DD-MON-YYYY).	The preferred method is to collect a complete end date. Partial dates (e.g., providing year only, month and year only) may be acceptable. Collecting this SAE-related data in the clinical database is the decision of the sponsor.	R/C	Y
What was the discharge diagnosis?	Hospital Discharge Diagnosis	N/A	N/A	N/A	N/A	HODDIAG	The clinician-determined condition of the subject at the time of release from the care facility	N/A	Record the discharge diagnosis.	Collecting this SAE-related data in the clinical database is the decision of the sponsor.	R/C	Y

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3 Introduction to Regulatory References

Data capture is part of the regulated activities in clinical trials. The following table lists the regulations and guidance that were referenced in developing the CDASH requirements for the SAE domain, including ICH guidelines and relevant regulations and guidelines from the European Commission (EC), the European Medicines Agency (EMA), and the US FDA.

The table lists sources that provide information on the collection, analysis, and reporting of safety data, including extended information required for expedited adverse event (AE) reporting. The table describes the kinds of information present in the regulations, but does not list individual data fields and does not include references to appropriate protocol designs for enabling safety assessments.

The Source column identifies the regulatory body issuing the regulation or guideline; the Regulation/Guideline column provides the title of the regulation or guideline; and the Description/Wording column provides a brief interpretation of the regulation or guideline as it applies to the collection, analysis, and reporting of safety data, as well as specific wording from the document where useful. Readers are advised to reference the original document for details.

In addition to the sources cited in the table and in other areas of this document, referenced sources include:

- US Code of Federal Regulations (CFR)
- European Commission and European Union directives and regulations[5-10]
- ICH Harmonized Efficacy Guidelines[11]
- FDA policy and procedure and compliance manuals and standards[12-14]
- FDA guidance for industry and investigators[15-17]

Notes:

- There are often several ways of interpreting guidance and regulations and, therefore, the information provided here should not be taken as an official or FDA/ICH-approved interpretation.
- Specific legislation at local country levels is not reflected and may need to be taken into consideration, depending on the countries and regions in which projects based on this CDASH guideline are planned to be conducted.

Source	Regulation/Guideline	Description/Wording
CFR	21 CFR § 312 Investigational New Drug Application[15]	Section 312.32 focuses on safety reports following investigational new drug (IND) submissions. It defines *serious* and the terms that are used in the definition (including *associated, expected*, and *unexpected* adverse drug reactions), and describes what reports should contain.
CFR	21 CFR § 314 Applications for FDA Approval to Market a New Drug[16]	Section 314.80 provides a definition of SAE.
CFR	21 CRF § 803 Medical Device Reporting[17]	Section 803.32 provides a list of variables to be collected and reported by user facilities for individual AE reports related to medical devices.
		Section 803.42 provides a list of AE-related variables that must be reported for individual safety reports by importers of medical devices.
		Section 803.52 provides a list of variables to be collected and reported by device manufacturers for individual AE reports related to medical devices
EC	Detailed Guidance on the Collection, Verification and Presentation of Adverse Reaction Reports Arising from Clinical Trials on Medicinal Products for Human Use (ENTR/CT3)[5]	This guidance presents extensive information on expedited reporting of serious AEs and is very similar to ICH E2A and E2B, although it provides somewhat more specificity in places. It does not define requirements for any additional data, but rather clarifies roles and responsibilities and timelines for reporting.
EC	EudraLex - Volume 10 - Clinical Trials Guidelines[8]	Chapter II provides detailed guidance on the collection, verification and presentation of adverse event/reaction reports arising from clinical trials on medicinal products for human use ("CT-3").

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Source	Regulation/Guideline	Description/Wording
EMA	Guideline on Good Pharmacovigilance Practices (GVP) Module VI[18]	Module VI provides guidance on collection, management, and submission of reports of suspected adverse reactions to medicinal products.
FDA	Submitting Study Datasets for Vaccines to the Office of Vaccines Research and Review[19]	This technical specifications document provides the sponsor/applicant with detailed information and specifications for the content of datasets submitted to FDA's Center for Biologics Evaluation and Research (CBER) Office of Vaccines Research and Review (OVRR) and is designed to aid clinical and statistical reviewers in the review of vaccine applications (e.g., biologics license applications).
FDA	Drug-Induced Liver Injury: Premarketing Clinical Evaluation[20]	See Section IV: Clinical Evaluation of DILI—for subjects who meet Hy's law criteria (altered liver function), additional data must be collected. Section C lists the CRF variables that must be collected.
ICH	Clinical Safety Data Management: Definitions and Standards for Expedited Reporting (E2A)[21]	Section II, Definitions and Terminology Associated With Clinical Safety Experience, provides definitions for terms commonly used in safety data reporting, such as *adverse event*, *serious adverse event*, and *expectedness.* By referencing the information to be submitted, the ICH suggests a minimal set of variables to capture the key information. The document • defines processes for expedited reporting, including what must be reported and how to determine reporting timeframe; and • outlines assessing safety during blinded treatment, associated with placebo treatment, and post-study events. The document does not go into any detail around individual data points required, although it contains some inferences about SAE narrative content. The description of the information required for expedited reporting of SAEs outlines further information that may be required to characterize deaths (e.g., allergy, drug or alcohol abuse, family history, findings from special investigations). Note that per ICH E2A, an autopsy or other postmortem findings must be included when available. This may have implications for data to be collected in every study.
ICH	Structure and Content of Clinical Study Reports (E3)[22]	Section 9 discusses rating AEs in terms of severity or relationship to drug. It also states that the report should define how consistency in applying the ratings was achieved between sites. Be clear in defining the severity and relationship categories, and include the definitions in CRF completion guidelines. Section 12.2 provides description of the kinds of safety summaries that must be conducted for AEs. These include summarizing AEs by body system, by intensity if used, by relationship to treatment if collected, and by treatment emergence. Summaries should include lab findings and vital signs changes identified as AEs. Even if AEs are categorized by relationship and/or treatment emergence, all AEs should be included in the summaries. Although E3 does not require that relationship to study drug be captured, the EU Directive on AEs[5] does require it. There must be a way to determine treatment emergence (both for increases in severity and in frequency). Labs and vital signs must be mergeable with AEs data, or somehow accessible. Section 12.2.3 describes the general analysis approach for AEs, including examination of relationship to dosage level if that seems appropriate. Section 12.2.4 provides a list of variables that must be included in AE listings (i.e., that need to be collected). These include typical AE variables, as well as study drug treatment data and concomitant treatment data and assessment of seriousness. Note that although the FDA does not require listings of AE data if AE data have been provided electronically, ICH guidelines do request these. Section 12.3 discusses the display of deaths and other SAEs: These should be discussed separately in the report, but essentially the same data are required for display. Deaths occurring both during the study as well as during the post-treatment follow-up period are to be included. The guideline includes a description of what the subject narrative must discuss, including a list of data points. In the list of appendices, the guidelines indicate that CRFs for subjects who died must be submitted. This implies that death data must be either collected for all studies on CRFs or on the serious AE collection instruments. In addition, it requires that "significant" AEs (i.e., AEs that were not serious, but required some significant concomitant therapy or intervention) be split out.

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Source	Regulation/Guideline	Description/Wording
		This implies that AEs requiring significant concomitant treatment (either pharmacological or non-pharmacological) be specifically identifiable.
ICH	E6 (R2) Guideline for Good Clinical Practice[23]	E6 (R2) discusses approaches to clinical trial design, conduct, oversight, recording, and reporting, as well as updated standards regarding electronic records and essential documents. Section 1.2, Glossary, defines adverse event. Section 4.11, Investigator, Safety Reporting, describes investigator responsibilities for reporting safety issues to sponsors, specifically deaths, other SAEs, lab AEs and AEs of special interest in a particular protocol. Section 5.16, Safety Information, outlines sponsor responsibilities for ongoing review of safety information; notification of investigators. Section 5.17, Adverse Drug Reaction Reporting, spells out the sponsor's regulatory responsibility to report all serious and unexpected AEs to IRBs, investigators, and regulatory authorities in accordance with ICH E2A. (Section 5 also describes sponsor responsibilities of periodic safety updates to regulatory authorities.) Most of the rest of the GCPs have more to do with actions relating to data, rather than the data themselves.
ICH	E9 Statistical Principles for Clinical Trials[24]	This guideline is fairly general in its observations, but provides some insight into regulatory expectations. Section VI, Evaluation of Safety and Tolerability, contains considerable discussion about appropriate approaches to the analysis and reporting of safety data, including summarizing by severity, start and duration of AE, and potential subpopulation analyses (e.g., sex, age). It also provides a definition for *treatment emergence*. Section VII, Reporting, supplements information contained in E3.
ICH	E19 Optimisation of Safety Data Collection - Draft Guidance[25]	This guideline describes an approach to safety data collection in some late-stage pre-approval or post-approval studies when the safety profile of a drug is sufficiently characterized. Per this guidance, a selective approach may improve the efficiency of clinical studies while reducing the burden to study participants.
NCI	Adverse Event Reporting Requirements[26]	This National Cancer Institute document provides definitions for terms used in reporting AEs, and lists the information that should be included when reporting different kinds of AEs resulting from different kinds of treatments for cancer (e.g., marketed, preregistration). These requirements are somewhat different from those for other types of diseases, due in part to the severity of the disease and the toxicity of the treatments. Among other information, the document includes definitions for AE grades that are used in oncology in place of severity assessments.

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4 Appendices

Appendix A: CDASH Serious Adverse Events Supplement Development Team

Name	Affiliation
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Appendix B: Data Elements from ICH E2B R2/R3

The following table provides a high-level comparison of the data element changes between ICH E2B (R2) and ICH E2B (R3).

Section A	E2B (R3)	Section A	E2B R2
C.1	Identification of the Case Safety Report	A.1	Identification of the case safety report
C.2	Primary Source(s) of Information	A.2	Primary source(s) of information
C.3	Information on Sender of Case Safety Report	A.3	Information on sender and receiver of case safety report
C.4	Literature Reference(s)		
C.5	Study Identification		
Section B		**Section B**	
D	Patient Characteristics	B.1	Patient characteristics
E	Reaction(s)/Event(s)	B.2	Reaction(s)/Event(s)
F	Results of Tests and Procedures Relevant to the Investigation of the Patient	B.3	Results of tests and procedures relevant to the investigation of the patient
G	Drug(s) Information	B.4	Drug(s) information
H	Narrative Case Summary and Further Information	B.5	Narrative case summary and further information

The following table provides a comparison of ICH E2B (R2) and ICH E2B (R3) fields relevant to the CIOMS form.

CIOMS form	ICH-E2B field (R2)	ICH-E2B field (R3)
REACTION INFORMATION		
1. Patient initials (first, last)	B.1.1 Patient (name or initials)	D.1 Patient (name or initials)
1. A Country	A.1.2 Identification of the country where the reaction/event occurred	E.i.9 Identification of the Country Where the Reaction / Event Occurred
2. Date of birth (day/month/year)	B.1.2.1 Date of birth	D.2.1 Date of Birth
2.A Age (years)	B.1.2.2 Age at time of onset of reaction/event	D.2.2 Age at Time of Onset of Reaction / Event
2. Sex	B.1.5 Sex	D.5 Sex
4-6. Reaction onset (day/month/year)	B.2.i.4 Date of start of reaction/event	E.i.4 Date of Start of Reaction / Event
7. Describe reaction(s)	B.2.i.0 Reaction/event as reported by the primary source B.2.i.8 Outcome of reaction/event at the time of last observation B.4.k.16 Action(s) taken with drug B.5.1 Case narrative including clinical course, therapeutic measures, outcome and additional relevant information	E.i.1 Reaction / Event as Reported by the Primary Source E.i.7 Outcome of Reaction / Event at the Time of Last Observation G.k.8 Action(s) Taken with Drug H.1 Case Narrative Including Clinical Course, Therapeutic Measures, Outcome and Additional Relevant Information
13. (including relevant test lab data)	B.3.2 Results of tests and procedures relevant to the investigation	F.r.3.4 Result Unstructured Data (free text)
8-12. Check all appropriate to adverse reaction	A.1.5.1 Serious - at case level	
Patient died	A.1.5.2 Seriousness criteria - at case level	E.i.3.2 Seriousness Criteria at Event Level
Involved or prolonged inpatient hospitalization		

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CIOMS form	ICH-E2B field (R2)	ICH-E2B field (R3)
Involved persistence or significant disability or incapacity		
Life threatening		
SUSPECT DRUG(S) INFORMATION		
14. Suspect drug(s) (include generic name)	B.4.k.1 Characterization of drug role B.4.k.2 Drug identification B.4.k.3 Batch/lot number	G.k.1 Characterization of Drug Role G.k.2 Drug Identification G.k.4.r.7 Batch / Lot Number
15. Daily dose(s)	B.4.k.6 Dosage text	G.k.4.r.8 Dosage text
16. Route(s) of administration	B.4.k.8 Route of administration	G.k.4.r.10 Route of Administration
17. Indication(s) for use	B.4.k.11 Indication for use in case	G.k.7 Indication for Use in Case
18. Therapy dates	B.4.k.12 Date of start drug	G.k.4.r.4 Date and Time of Start Drug
(from/to)	B.4.k.14 Date of last administration	G.k.4.r.5 Date and Time of Last Administration
19. Therapy duration	B.4.k.15 Duration of drug administration	G.k.4.r.6 Duration of Drug Administration
20. Did reaction abate after stopping drug? Yes/No/Na	B.4.k.16 Action(s) taken with drug	G.k.8 Action(s) Taken with Drug
21. Did reaction reappear after reintroduction? Yes/No/Na	B.4.k.17.1 Did reaction recur on re-administration?	G.k.9.i.4 Did Reaction Recur on Re-administration?
III. CONCOMITANT DRUG(S) AND HISTORY	B.4.k.1 Characterization of drug role	G.k.1 Characterization of Drug Role
22. Concomitant drug(s) and dates of administration (exclude those used to treat reaction)	B.4.k.2 Drug identification B.4.k.12 Date of start of drug B.4.k.14 Date of last administration	G.k.2 Drug Identification G.k.4.r.4 Date and Time of Start of Drug G.k.4.r.5 Date and Time of Last Administration
23. Other relevant history (e.g. diagnostics, allergics, pregnancy with last month of period, etc.)	B.1.7 Relevant medical history and concurrent conditions (not including reaction/event)	D.7.2 Text for Relevant Medical History and Concurrent Conditions (not including reaction / event)
MANUFACTURER INFORMATION		
24.a Name and address of manufacturer	A.1.11.1 Source(s) of the case identifier (e.g. name of the company, name of regulatory agency)	C.1.9.1.r.1 Source(s) of the Case Identifier
24.b MFR control no.	A.1.11 Other case identifiers in previous transmissions	C.1.9 Other Case Identifiers
24.c Date received by manufacturer	A.1.7b Date of receipt of the most recent information for this report	C.1.5 Date of Most Recent Information for This Report
24.d Report source Study Literature Health professional	A.1.4 Type of report A.2.2 Literature reference(s) A.2.1.4 Qualification	C.1.3 Type of Report C.4.r.1 Literature Reference(s) C.2.r.4 Qualification
25.A Report type Initial Follow-up	Transferring of correct dates (from 24.c) is based on the CIOMS Report type, i.e. if the report is initial or a follow-up.	
Date of this Report		

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Appendix C: Glossary and Abbreviations

The following table lists abbreviations and terms used in this document. Additional definitions can be found in the CDISC Glossary (available athttps://www.cdisc.org/standards/foundational/glossary/).

Term / Abbreviation	Definition
AE	Adverse event: Any untoward medical occurrence in a patient or clinical trial subject administered a medicinal product and which does not necessarily have a causal relationship with this treatment.[6,8,21]
AESI	Adverse event of special interest
AR	Adverse reaction: An untoward and unintended response to an investigational medicinal product related to any dose administered.[6,8,21]
Case	An observation requiring investigation, which includes problems that might or might not involve individual or groups of investigative subjects.[27]
CBER	(US FDA) Center for Biologics Evaluation and Research
CDASH	Clinical Data Acquisition Standards Harmonization Project: The name for the project that delivers basic data collection fields (this document).
CDISC	Clinical Data Interchange Standards Consortium, a Collaborative Group Member
CDM	Clinical data management
CDMS	Clinical data management system
CFR	(US) Code of Federal Regulations
Collected	Within this document, "collected" refers to information that is recorded and/or transmitted to the sponsor. This includes data entered by the site on CRFs/eCRFs as well as vendor data such as core lab data. This term is a synonym for “captured.”
Controlled terminology	A finite set of values that represent the only allowed values for a data item. These values may be codes, text, or numeric. A codelist is one type of controlled terminology.
CRF	Case report form (sometimes "case record form") A printed, optical, or electronic document designed to record all required information to be reported to the sponsor for each trial subject.
CTCAE	Common terminology criteria for adverse events
Dataset	A collection of structured data in a single file
Derived	Within this document, "derived" refers to information that is not directly entered into the specific data field by the investigator site or by a core lab. This category includes auto-encoded data, calculated data, and similar electronically generated data, but not prepopulated fields.
Domain	A collection of observations with a topic-specific commonality about a subject
Drug safety	The group of persons at an institution, organization, or company who oversee the collection, detection, assessment, monitoring, and reporting to appropriate authorities of adverse effects with pharmaceutical products.
eCRF	Electronic case report form
E2B message	Electronically transmittable ICSR
EDC	Electronic data capture
EMA	European Medicines Agency; formerly EMEA
EMEA	European Medicines Evaluation Agency
ERC	Ethics review committee; see IRB
FDA	Food and Drug Administration, part of the US Department of Health and Human Services
GCP	Good clinical practice
HL7	Health Level Seven International (organization)

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Term / Abbreviation	Definition
ICH	International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use
ICH E2A	ICH guidelines on Clinical Safety Data Management: Definitions and Standards for Expedited Reporting
ICH E2B	ICH guidelines on Clinical Safety Data Management: Data Elements For Transmission Of Individual Case Safety Reports
ICH E3	ICH guidelines on Structure and Content of Clinical Study Reports
ICH E6 (R1)	ICH Guideline for Good Clinical Practice
ICH E9	ICH guidelines on Statistical Principles for Clinical Trials
ICSR	Individual case safety report: Complete information provided by a reporter at a certain point in time to describe an event or incident of interest. The report can include information about a case involving one subject or a group of subjects.
IND	Investigational new drug. An IND application is required by the FDA before clinical trials of a new drug or new biological agent may be initiated.
IRB	Institutional review board (in some countries, ERC): An appropriately constituted group that has been formally designated to review and monitor biomedical research involving human subjects
ISO 8601	International Organization for Standardization document of character representation of dates, date/times, intervals, and durations of time
MedDRA	Medical Dictionary for Regulatory Activities
MHLW	Ministry of Health, Labour and Welfare (Japan)
NCI	(US) National Cancer Institute, part of NIH
NCI: EVS	NCI) Enterprise Vocabulary Services
NIH	(US) National Institutes of Health
OVRR	(US FDA CBER) Office of Vaccines Research and Review
PI	Principal investigator
Preprinted (pre-printed)	Items that are part of the original printing on a paper CRF (e.g., the unit required for a response, such as “years” for an age question). These data may or may not be stored in the database.
Prepopulated (pre-populated)	Items that are part of the eCRF (or data collection device) that are not enterable/modifiable (see also *preprinted).* These data are stored in the study database.
SAE	Serious adverse event (also "serious adverse reaction"): Any untoward medical occurrence or effect that at any dose results in death, is life-threatening, requires hospitalization or prolongation of existing hospitalization, results in persistent or significant disability or incapacity, or is a congenital anomaly or birth defect).[21]
SDTM	Study Data Tabulation Model
SDTMIG	SDTM Implementation Guide
SNOMED	Systematized Nomenclature of Medicine Clinical Terms
SUSAR	Suspected unexpected serious adverse reaction: An adverse reaction, the nature or severity of which is not consistent with the applicable product information (e.g. investigator's brochure for an unauthorized investigational product or summary of product characteristics for an authorized product).[21]
Suspect drug	The intervention that is attributed as to being the cause of the SAE
Study treatment	The drug, device, therapy, or process under investigation in a clinical trial which has an effect on outcome of interest in a study (e.g., health-related quality of life, efficacy, safety, pharmacoeconomics). Synonyms: *intervention, therapeutic intervention, medical product*.
WHO	World Health Organization
WHO ART	World Health Organization Adverse Reaction Terminology

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Appendix D: References

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Appendix E: SAE Narrative

Content Elements for the Narrative

The objective of the SAE narrative is to summarize all relevant clinical and related information, including patient characteristics, therapy details, prior medical history, clinical course of the event(s), laboratory evidence, and any other information that supports or disproves a diagnosis for an adverse drug reaction. The information should be presented in a logical time sequence.

All information relevant to the case may not be available at the time of the report. However, it is expected that the site investigator/staff perform due diligence to obtain information that contributes to the full understanding and medical assessment of the case.

As per ICH E3,[22] a patient narrative should describe:

- The nature, intensity and outcome of the event
- Clinical course leading to the event
- Timing of study drug administration
- Relevant laboratory measures
- Counter measures
- Action taken with the study drug in relation to the event
- Postmortem findings (if applicable)
- Investigator's and sponsor's opinion on causality

Additionally, patient identifier, age, gender, clinical condition, disease being treated, relevant medical history, concomitant and prior medications should be included.

Flow of Narrative

- Protocol/Study ID: XXXX
- Study Title/Study description: Post-Marketing Surveillance of DRUG mg (ingredient) to Evaluate Its Safety and Efficacy.
- Screening number/ Randomization No: XXX-XX-XXX
- Patient ID/Subject ID: XXXX-XXXX
- Patient demographics
- Patient medical history and concomitant medication information
- Suspect product information timing and conditions surrounding the onset of the reaction(s)
- Medical device reporting (if applicable)
- Product unblinding product information (if applicable)
- Clinical course of the events, with an indication of timing of event corresponding to drug administration
- Nature, intensity/severity, and outcome of the event
- Relevant laboratory findings
- Treatment administered for the event
- Action taken with respect to the drug
- Dechallenge and rechallenge information, if applicable
- Postmortem findings (if applicable)
- Clinical relation of event with suspect drug

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- The investigators causality assessment
- The sponsor's medical evaluation and comment
  Additional information
- Product exposure during pregnancy (maternal/paternal)
- If fetus: provide [Gestational Age], (or mother's LMP), at time of event. Also, [Gestational Age/Trimester] at first drug exposure and duration of exposure. If birth, provide details of [Infant Status] at birth. If hospital stay is complicated, provide details of hospital stay.

Narrative Template

This narrative template is shared as an example of a complete narrative description of an individual case safety report (ICSR), single-case SAE report. ICSRs are a core component of pharmacovigilance. This structure allows for a complete understanding of the event/reaction and a causality assessment of the product. Please complete the narrative with the information suggested, and add further details if they are clinically relevant for the description of the SAE. Conversely, if there are sections that are not applicable to the case, these do not need to be included.

All information relevant to the case may not be available at the time of the report. However, it is expected that the site investigator/staff perform due diligence to obtain information that contributes to the full understanding and assessment of the case.

Narrative Example:

Protocol title (Insert protocol number): [Insert protocol title e.g. An open-label, multicenter clinical trial]

A clinical investigator reported a [Age] year old [Race], [Height and Weight, if relevant], [Male/Female] in [Study] who reported [Primary SAE] on [Date of SAE].

The patient's medical history of [history, including the duration of concurrent illness, age at diagnosis, date of diagnosis or onset date not reported].

Concomitant therapy included [generic name of relevant concomitant drugs with/without indication if appropriate. Or provide a general statement such as subject was also receiving multiple concomitant medications, with or without indications (for example, he/she was taking multiple medications for pain, hypertension and depression). Note that concomitant medications include those medications taken within a reasonable time frame (30 days) prior to the onset of an adverse event and medications taken by the subject at the time of the reported adverse event.]

The patient commenced treatment with [SUSPECT PRODUCT], dose, frequency, on date for indication.

On [date MM DDD YYYY], the subject presented with event [detailed event description including signs, symptoms and details about hospitalisation including prolongation of existing hospitalisation]. which required hospitalisation.

The patient's laboratory results on [DD MMM YYYY] were as follows: [list relevant physical findings, exams and laboratory results].

Corrective treatment for the event included [describe medications, procedures, tests, investigations etc.].

Action taken with suspect drug included (discontinued/temporarily stopped/dose decreased/dose increased/no change/unknown).

State outcome of event following treatment (recovered, unresolved, recovered with sequelae (describe sequelae), or death). Note if event resolved spontaneously.

Fatal patient outcome details: include date and cause of death, the timing of death in relation to the event onset and suspect drug therapy duration, include an autopsy or post-mortem findings when available.

The investigator causality assessment: The event of (USE MedDRA PT term) include causality assessment in relation to each suspect drug therapy) was related/ unrelated to study therapy. Include the rationale for the causality assessment.

Company causality assessment: The event of (USE MedDRA PT term) include causality assessment in relation to each suspect IMP drug therapy) was related/ unrelated to study therapy.

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